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Exploring the Development of Mental Health Training Programs for Black and Brown Female Teens and Preteens in Illinois Nonprofits

By

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DISSERTATION

Submitted in Partial Fulfillment of the Requirements

For the Degree of Doctor of Education, Ed.D.
Interdisciplinary Leadership, NPSE Concentration

Governors State University
University Park, Illinois

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by

Darryca Brim Mims

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Abstract

The spread of social media has posed severe mental health issues to Black and Brown female teens and pre-teens (BBFTPTs), as they are particularly prone to cyberbullying, racial discrimination, and Eurocentric beauty ideals. This qualitative analysis explored the incorporation of negative effects of social media by Chicago nonprofits on their mental health training programs for BBFTPTs. Employing semi-structured interview methodology on seven nonprofit leaders, the research examined organizational decision-making on curriculum development within the theoretical framework of resource dependency theory, institutional theory, and change management theory. Data analysis revealed funding limitations, pressure from external stakeholders on curricular decisions, and the post-pandemic organizational adjustment to the needs of mental support, all of which impact curricular choices. Some of the major conclusions show that organizations are working on new interventions, including media literacy programs, peer leader models, or culturally responsive interventions that can turn the youth into active creators of online content rather than mere consumers. The research found major changes in the traditional mental health paradigm to identity-based programs, which deal with the multi-faceted interplay between social media exposure, cultural identity, and mental health.

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CHAPTER I: INTRODUCTION

This chapter sets the research context and provides the research background knowledge regarding the effects of social media on the mental health of Black and Brown female teens and pre-teens (BBFTPTs). The chapter provides both the background of the problem under investigation regarding the overlap between social media exposure and mental health adversity in this high-risk group, especially regarding the prevailing socioeconomic disparities. It identifies the purpose of the research, which aims at examining how nonprofits in Illinois can design and roll out specialized mental health training curricula to overcome the adverse impacts of social media on BBFTPTs. It also outlines the research assumptions and limitations and provides definitions of relevant terms that serve as the foundational elements of the study.

Problem Background

In an era characterized by the predominance of online communication, social media platforms are increasingly considered as environments that foster connectivity among individuals, enabling the sharing of personal memories and experiences. These platforms are also viewed as vital tools for self-expression and the development of interpersonal relationships. Despite the numerous advantages associated with social media usage, it also poses significant risks to individual identity and overall well-being (American Psychological Association, 2022). Research indicates that increased exposure to social media can adversely affect mental health, particularly among children and adolescents, such as anxiety, depression, issues related to self-esteem, and increased alcohol use (American Psychological Association, 2022). The potential for negative consequences is even harder for adolescents of color, particularly BBFTPTs, who are often subjected to racial discrimination, social comparison, cyberbullying, and body shaming (Choukas-Bradley et al., 2022). Empirical evidence reveals that BBFTPTs develop mental health challenges due to negative experiences they encounter on social media, such as

cyberbullying, colorism, and the pressures laid on them to conform to Eurocentric standards of beauty (Tao & Fisher, 2021). Reports that more than 70% of Black and Latino youth are interacting with at least one social media platform (Tao & Fisher, 2021) highlight the importance of raising awareness about the impact of social media among youth and ensuring they have access to mental health services.

In addition, Beech et al. (2021) reported that the United States has one of the highest poverty levels among developed countries, with the most significant burden falling on people of color. The economic disparities were exacerbated by the COVID-19 pandemic, which fueled poverty and financial hardships, unmasking the vulnerabilities of minority groups, especially African Americans, who did not have access to insurance and basic health services (Reyes, 2020). Family challenges were continuously shared on social media, with BBFTPTs interacting with the content regularly, which resulted in negative health outcomes. This context highlights the interplay of socioeconomic disparities, increased social media exposure to hardship, and limited access to mental health services has created a particularly challenging landscape for BBFTPTs. Tao and Fisher (2021) highlighted that there is a critical need for targeted mental health interventions that address both the systemic issues and the specific social media-related challenges faced by this vulnerable population.

Young people are the responsibility of the community in which they live and go to school. One responsibility is ensuring that the mental health needs of the students are well addressed (Tam et al., 2024). School districts, as members of the greater community, should identify the needs of students and promptly and efficiently address them, including BBFTPTs, and ensure they receive appropriate services through in-school resources or referrals to specialized nonprofits (Chicago Public Schools, 2022). Nonprofits in Illinois could deliver specialized, culturally competent mental health services to BBFTPTs and address their unique

needs with tailored programs and trauma-informed approaches. They could complement school resources by leveraging community support and funding to provide comprehensive care. Additionally, they could create safe spaces for discussion and support, empowering girls to navigate social media pressures and cultivate a strong sense of identity and well-being.

Throuvala et al. (2021) found that educational interventions promoting critical media literacy and healthy online behavior can effectively mitigate the negative effects of social media use among adolescents. For example, nonprofits could provide workshops or programs specifically addressing the impact of social media, offering guidance on healthy digital habits, critical thinking skills, and self-care techniques to mitigate the negative effects of social media on mental health. Other potential programs implemented by nonprofits, as suggested by scholars, could include mental health training concerning social media for BBFTPTs to focus on building resilience, teaching coping strategies, and fostering positive self-esteem. The core principles of these interventions should be based on developing the understanding that resilience and coping skills are essential protective factors against mental health issues in adolescents, especially in the face of social stressors such as those encountered online (Sage et al., 2021).

There are several governmental services that address the impact of social media, especially in school settings. For example, the federal government runs a Substance Abuse and Mental Health Services Administration (SAMHSA) program that addresses mental health issues and raises awareness of the need to get help when facing mental health challenges. One of the projects sponsored by SAMHSA is Advancing Wellness and Resiliency in Education (AWARE), which works with educational agencies to increase awareness of mental health issues among school-going youth (SAMHSA, 2024). This program helps educational institutions detect and address mental health issues among students, including those caused by social media.

However, there is a lack of accurate data and a proper framework for addressing social media's impact, which poses a serious challenge for not only program managers but also policymakers in nonprofits and elsewhere. Congress needs to discuss current proposals, as highlighted by GovTech (2023), to build a proper framework that will address the social media issues and the most appropriate solutions. Without a proper educational framework, leaders lacking appropriate knowledge of the impact of social media could rely on information from web searches, which is not always scientifically certified to address the social media issue within society. Additionally, the lack of a proper framework also means that the government cannot ask for discussions on social media to be part of mental health services. Without these programs in place, it can be said that the current curriculum does not address the impact of social media due to the lack of standards in addressing the issue from the educational front.

Furthermore, while funding for mental health services for BBFTPTs involves multiple institutions, such as the City of Chicago grants, federal and state funds, and nonprofit grants and fundraising efforts (City of Chicago, 2024), these efforts are not sufficient to effectively address the mental health issues triggered by negative exposure to social media. Community organizations, parents, and other partners such as private sponsors and healthcare providers could contribute additional resources and support; however, these resources must be more thoroughly examined to ensure input from educational experts and psychologists in the development of curricula for mental health services. These efforts should also be guided by best practices and ethical standards set by professional organizations, such as the American Psychological Association (APA) and the National Association of Social Workers (NASW), which emphasize culturally competent care and equity in service delivery to provide for the needs of the population explored in this study.

Recognizing this concern, this study explored the intersection between social media use, mental health, and educational interventions, aiming to increase the understanding of how nonprofits in Illinois can develop a mental health training curriculum to equip BBFTPTs with the necessary tools to navigate the digital space safely. Along with exploring how Illinois-based nonprofit training programs could be implemented to empower BBFTPTs to cope with the negative impacts of social media, the study also investigated the factors influencing the successful incorporation of educational materials and activities related to social media's negative impact in the mental health training curriculum.

Purpose of Research and Intended Audience

The purpose of this research is to explore the mental health services currently delivered by the Illinois nonprofits and explain the context and the conditions under which nonprofits could focus their training curriculum to ensure it addresses the negative impacts of social media on the mental health of BBFTPTs. The intention was to guide policy and strategy formulation on ways to create a nonprofit curriculum to address mental health issues faced by BBFTPTs for long-term positive outcomes. The research targeted educators and nonprofit institutions with access to BBFTPTs by educating them on ways they can implement interventions and develop curriculum to address mental health issues caused by social media. Moreover, the research targeted policymakers, as it could serve as a roadmap for policy formulation to ensure that appropriate resources are made available to support the affected population.

While existing research has explored the link between social media and mental health, there are very few studies focusing specifically on how to address the effects of social media on preteen and teenage girls of color. This research addressed this by providing insights into the unique challenges this group faces concerning social media influences. Tao and Fisher (2021) highlighted that little is known about the existing relationship between racial justice activities

and exposure to social media racial discrimination among youth of color. The point here is that while social media can empower young people of color to engage in activism, it can also expose them to increased levels of racial discrimination perpetuated by social media, which can potentially harm their mental health (Mady et al., 2023). This research intends to raise awareness concerning how nonprofits can address the negative influence of social media experienced by BBFTPTs. The study findings can be used to help nonprofits develop effective policies and teaching curricula strategies that will help protect this minority population from developing mental health challenges associated with exposure to the negative effects of social media.

The main research stakeholders include the government, donors, nonprofits, and community-based organizations. The government can influence policy development and ensure proper frameworks are set up to guide intervention for BBFTPTs facing mental health issues due to social media. The government can also fund effective programs to increase awareness of mental health issues and develop strategies to address the existing challenges. Community-based organizations could also carry out a curriculum review of current school programs to assess the impact of social media and how it impacts students' well-being.

Problem Statement

Nonprofits in Illinois do not meet the needs of BBFTPTs in addressing the adverse effects of social media on this portion of the population. As already noted, there are rising numbers of depression, anxiety, and suicide (DAS) among preteens and teens, particularly BBFTPTs, due to social media usage (Choukas-Bradley et al., 2022). Federal data indicates that the rate of suicide among Black youth has surpassed that of White youth. Currently, the rate of suicide among Black youth stands at 54%, which is a significant increase since 2018 (Akkas & Corr, 2024). Among brown youths, research in 2019 found that about 17% of Brown youths had seriously contemplated suicide, with the rate for girls being almost 1 in 4 at 23% (Reynolds,

2022). The COVID-19 pandemic exacerbated mental health issues among youth from minority groups as they were exposed to too much discrimination and bias. Mayne et al. (2021) stated that the pandemic saw a significant rise in suicidal attempts among adolescents, which underscored the importance of suicidality and depression screening.

In response to this crisis, some nonprofits have stepped up to provide mental health training programs explicitly targeting BBFTPTs (Throuvala et al., 2021). However, there is a handful of programs that deal explicitly with the complexities of social media targeted at youth in the Chicago area and elsewhere, and how to mitigate its negative impact on their mental health. While these are fundamental services, the kind of offering currently provided by a number of non-profits does not lend specificity to make any real difference in the unique challenges BBFTPTs face with social media's impact on mental health. These are generally broad services targeting just about anybody and may lack the peculiar cultural and socio-economic factors that disproportionately affect minority groups. As a result, the specific needs of BBFTPTs, which put them, particularly, at risk for issues such as online racial discrimination, body image insecurities founded upon Eurocentric beauty standards, and systemic injustices, remain poorly served.

Without refinements in targeting mental health services, such provision of mental health for these young people is often sparse, making them more vulnerable to threats of depression, anxiety, and other adverse outcomes related to their social media experiences (Throuvala et al., 2021). Unless there is development regarding specialized programs that can deal directly with specific needs, BBFTPTs might see worsening mental health outcomes, including increased rates of depression, anxiety, and suicidal thoughts (Murthy, 2024). The broader, nonspecific nature of the interventions being performed could only manage to continue to perpetuate the

existing disparities by leaving vulnerable populations still underserved and unsupported in the very areas where they are at greatest risk.

Research Question

The present research attempted to bring to the fore a lack of services from the Illinois nonprofits and emphasize the urgent need for tailored interventions that can help mitigate the adverse impacts of social media on this particularly vulnerable demographic segment. More specifically, it sought to understand how mental health training programs and curricula concerning social media-related mental health challenges faced by BBFTPTs can be developed in Illinois. The research was founded on the following research question:

> How do Chicago nonprofits that offer mental health services for youth, particularly for Black and Brown females, integrate the negative influence of social media topics in their educational training curricula?

Assumptions

Various assumptions formed the foundation of this study and influenced the research design and methodology. The initial assumption was that the participants would interview in honesty and openness, offering true insight into their organizational practices and experiences with mental health training programs in BBFTPTs (Ruslin et al., 2022). The research operated under the assumption that the leaders of nonprofits had the knowledge and experience to effectively reflect how they addressed the effects of social media among this at-risk population. It was also presumed that the theoretical framework of change management theory, institutional theory, and the resource dependency theory best accounted for the multifaceted organizational factors that shaped the curriculum development and implementation process within nonprofits in Illinois (DiMaggio & Powell, 1983; Pfeffer & Salancik, 2015).

The study assumed that semi-structured interviews and program analysis would provide valid data to permit the study to comprehend factors affecting the integration of educational training curricula. Moreover, purposive sampling took into consideration a wide range of opinions of stakeholders of interest, which may have limitations in generalizing the results beyond the Illinois setting (Campbell et al., 2020). Lastly, it is presumed that the objectivity of the researcher remained intact during the data collection and analysis stages, but it was assumed that pre-knowledge and experiences could have seeded subtle biases that were addressed by triangulation and member checking methods.

Limitations

Several limitations in this study need to be considered. The study used a small, purposive sample size of nonprofit organizations that were only based in Illinois, which resulted in a limitation of the generalizability of results to any other geographical areas or situations. The regional focus also did not adequately reflect the heterogeneity of nonprofit practice across states, especially the difference between urban and rural areas or across policy and financing contexts. Additionally, the study timeline, conducted from February 2025 to May 2025, represents a focused timeframe that may have influenced the scope of data collection and analysis opportunities available to the research. Moreover, the study used interviews with single respondents representing each organization, mostly leadership staff, which presented a limitation in the extent to which organizational views were covered. Although these participants were well educated concerning curriculum development, they could not express the thoughts of direct service providers, mental health practitioners, or the youths affected.

The study relied upon self-reported data, which was prone to social desirability bias where study participants favorably portrayed their organizations. Implementation practices and

program impact claims could not be verified independently without observational or documentary verification methods. Moreover, the theoretical framework, a combination of change management theory, institutional theory, and resource dependency theory, may have limited or altered the researcher's results in comparison to interpreting the results obtained by each theory singly or in using other potential frameworks like critical race theory or ecological systems theory. Despite these shortcomings, the research offered contextually rich insights that added to an appreciation of how nonprofits can become innovative when it comes to responding to mental health issues that face Black and Brown young women in a digital environment.

Delimitations

In this study, certain delimitations were developed that expressed conscious methodological and contextual decisions aimed at ensuring focused and manageable research scope. Geographically, their study focused on Illinois-based nonprofits, notably in providing services to communities in Chicago, as opposed to analyzing a national-level study that would have introduced too much variability in state policies, funding mechanisms, and social contexts (Akkas & Corr, 2024). This deliberate narrowing of the population of BBFTPTs compared to studying all youth populations tends to dilute the specificity of findings (Mady et al., 2023) and reflects the specific vulnerabilities of this segment of the population to social media-related mental health concerns. Organizationally, the research solely aimed at nonprofits, omitting government and private organizations, because nonprofits are the leading providers of community-based interventions to this vulnerable group in urban communities around Illinois (Throuvala et al., 2021). Methodologically, it pursued a qualitative approach with semi-structured interviews instead of quantitative methodologies, which allowed an in-depth analysis of complex social dynamics and lived experiences of curriculum integration (Ruslin et al., 2022).

The theoretical approach was restricted to change management theory, institutional theory, and the resource dependency theory, which together touched on organizational adaptation, systemic barriers, and resource constraints without considering other upcoming relevant frameworks such as social cognitive theory or systems theory. The collection of data was limited to interviews with seven nonprofit leaders instead of the direct involvement of BBFTPTs themselves, and this was partially because of the ethical concerns about vulnerable minors and the difficulty of informed consent (Mwita, 2022). These exclusions allowed the study to be focused on the problem of how Illinois nonprofits could design specific curricula to help overcome the adverse effects of social media on the mental health of BBFTPTs and be methodologically rigorous and ethically correct.

Definition of Terms

BBFTPTs (Black and Brown Female Teens and Pre-Teens) — African American and Latina girls aged 10-19 disproportionately affected by social media's negative mental health impacts (Agonafer et al., 2021).

Change Management Theory — Lewin's three-stage (unfreeze-change-refreeze) model that explains how nonprofits can redefine their Mental health programming strategies (AlManei et al., 2018).

Colorism — Discrimination privileging lighter skin tones, contributing to body image issues among Black and Brown teens through Eurocentric beauty standards (Elmi, 2024).

Digital Resilience — The ability of the youth to safely navigate online spaces as well as the ability to maintain good identity construction, despite exposure to unhealthy digital content (Livingstone & Third, 2017).

Institutional Isomorphism — DiMaggio and Powell's concept explaining how nonprofits transform into each other due to the pressure of conformity with current organizational norms (DiMaggio & Powell, 1983).

Mental Health Training Programs — Culturally responsive interventions designed by nonprofits aimed at assisting with the psychological needs of Black and Brown female teens (Throuvala et al., 2021).

Resource Dependency Theory (RDT) — Pfeffer's and Salancik's framework of investigating the effects and impact of external-funding dependencies in programming needs and organizational flexibility (Pfeffer & Salancik, 2015).

Single Session Interventions (SSIs) — Brief and stand-alone therapeutic interventions that offer instant mental support and skill development during a single-contact session (Schleider et al., 2020).

Social Cognitive Theory — Psychological theory suggesting that individuals learn behavior and attitudes through observation, imitation, and modelling in social settings (Bandura, 1986).

Social-Emotional Learning (SEL) — Programming that provides emotional skills building as part of the service provision, such as non-mental health-oriented programming like literacy or math (Bittner, 2015).

Trauma-Informed Care — Mental health programming model that acknowledges trauma experiences by designing safe, supportive spaces that foster healing and avoid re-traumatization (Ranjbar et al., 2020).

Chapter Summary

Mental health issues among BBFTPTs have been on the rise in recent years, with the COVID-19 pandemic exacerbating the issue, mainly due to preexisting financial challenges

among minority groups. Social media exposed the population to discrimination and bias during the pandemic, which increased suicide cases, especially among BBFTPTs. Despite social media's impact on the population, curricula in schools and at nonprofits in Illinois do not address its impact due to a lack of awareness and a lack of a policy framework that supports its introduction. Key stakeholders, including the government, community-based organizations, and mental health professionals, have a fundamental role to play in dealing with the impact of social media on mental health to ensure BBFTPTs get access to essential services that help them deal with mental health issues. While Illinois nonprofit organizations offer services to BBFTPTs, the effects of social media are frequently not addressed, leaving the specific needs of these vulnerable groups unaddressed. This research attempted to close that gap.

The next chapter covers an in-depth literature review, undertaking the historical progression of social media and how it has been documented to negatively affect mental health, specifically in young people of color, along with the current mental health services, training programs, and the theories that inform intervention solutions to combat these issues

CHAPTER II: LITERATURE REVIEW

Chapter 2 offers an extensive literature review of the study investigating a complicated relationship between social media usage and mental health in Black and Brown female teens and pre-teens of color (BBFTPTs). This chapter is organized into four key sections exploring systematically the curation of research and theories available to date. The first portion provides a topical overview tracing the historical development of social media, its disparate effects on children of color, and how the COVID-19 pandemic has worsened prevalent social issues and mental health disparities. The second section lays down the theoretical groundwork with three main frameworks of change management theory, institutional theory, and resource dependency theory that, when combined, form the cornerstone of how nonprofit organizations can best help people deal with these challenges. The last section examines literature themes on government and the private sector response to the influence of social media, mental health training programs to address problems caused by social media, and service design and delivery by the nonprofit sector. Lastly, the final section concludes with these results to outline the uncertainties of the problem and the necessity of interventions to overcome the specific uncertainties of BBFTPTs in social media mental health effects.

Research Exploring Social Media Influence on Mental Health

This section presents a review of research examining the historical development of social media, its applications, and the adverse consequences it poses, particularly for children of color and their mental health. Furthermore, it explores how the negative impacts of social media have been exacerbated by the COVID-19 pandemic.

Historical Background

The roots of social media trace back to the early days of the internet, with platforms like SixDegrees in the late 1990s paving the way for the interconnected digital landscape we know

today (Chayko, 2020). The early 2000s witnessed the rise of platforms such as MySpace and Friendster, which further solidified the concept of online social networking. With the advent of Web 2.0, these platforms evolved from simple communication channels to interactive spaces for social interaction. The subsequent explosion of platforms like Facebook, YouTube, and Twitter marked a turning point, making social media an integral part of daily life for millions worldwide. Research into the effects of social media has paralleled this growth, examining its impact on various facets of life, from mental health and relationships to political engagement and consumer behavior. Studies specifically focusing on social media's influence in schools have revealed a mixed bag of results. While some research suggests that social media can facilitate collaborative learning and enhance student engagement, other studies raise concerns about its potential to distract from academic tasks and contribute to cyberbullying (McCrory et al., 2020; Naslund et al., 2020).

A key distinction lies in the differential impact of social media on children and adults. Young people, often dubbed 'digital natives,' have grown up in a world where social media is ubiquitous. This constant exposure can make them particularly vulnerable to the negative effects of online interactions, including cyberbullying, social comparison, and body image issues. In contrast, adults, having experienced a pre-social media world, may possess greater digital literacy and resilience, potentially mitigating some of the adverse effects (Kross et al., 2021). Research also points to unique challenges for marginalized groups, including BBFTPTs, who may face heightened exposure to online racism and discrimination. This points to the importance of understanding not only the general effects of social media but also its nuanced impact on specific demographic groups. This chapter explored in great detail these complexities, exploring the existing research on social media, its historical evolution, its impact in educational settings, and the unique challenges faced by young users, particularly Black and Brown girls. By

understanding this broader context, we can better appreciate social media's specific mental health implications for this vulnerable population and develop effective nonprofit interventions to address these challenges.

Social Media Types and Use

Social media can be defined as collective websites or applications that are focal on communication, interaction, collaboration, and community-based inputs. There are a variety of social media platforms, and the way in which messages are given and received in multimedia modalities differs. Some popular social media applications include TikTok, Instagram, YouTube, Facebook, X (formerly Twitter), and Telegram. Presently, Facebook is the largest platform based on active users. However, platform popularity is influenced by the interests and age of users. The aforementioned Facebook, for example, skews toward older demographics. Young people, however, use social media at a 97% frequency rate (Naslund et al., 2020). Young people additionally reported a high frequency of negative effects from social media that included exposure to harm, social isolation, depressive symptoms, and bullying (Naslund et al., 2020). Studies have consistently shown that greater frequency of social media use contributes to increased risk for a variety of mental health symptoms and poor well-being, with the most problematic consequences among young people (Marciano et al., 2022; Naslund et al., 2020).

Problems with adolescent social media use include social comparison, fear of missing out, and exposure to harmful content (Marciano et al., 2022). Bozzola et al. (2022) found that negative aspects of social media use in children and adolescents, as represented in research, primarily dealt with issues of depression, diet, and psychological problems specifically. Their meta-analysis identified additional problems for young people, like sleep issues, addiction, anxiety, sex-related elements, behavioral issues, body image issues, and even vision problems. Regarding teen girls, a study conducted by Vogels and Gelles-Watnick (2023) for the Pew

Research Center concluded that they were more likely to report negative experiences and the impact of social media than boys. In general, social media collects a great deal of data related to users, and this data can be used to examine social phenomena related to platforms. In this capacity, the identified issues of depression and suicide have resulted in calls for big data to be used to identify signs of mental disorders that could need attention, like depression and suicide (Skaik & Inkpen, 2020). Not all social media use by adolescents is harmful. For example, a data meta-analysis has shown that some types of social media use among adolescents, like one-to-one communication and self-disclosure, can combat isolation, reduce feelings of loneliness and stress, and forge stronger peer relationships (Marciano et al., 2022). Therefore, categorizing social media use as good or bad would be erroneous and oversimplifies complex variables.

Social Media and Mental Health Among Young People

Mental health issues occur at statistically significant levels in young people, with Tam et al. (2024) establishing that 10% of children and adolescents globally experience some type of mental disorder. While it would be expected that as a group born with social media, they would better acclimatize to it, young people are significantly affected by social media. Among the mental health issues facing this age group, depression, anxiety, and suicide are the leading issues impacting the group, with most sufferers not seeking care for these issues (Tam et al., 2024). Many mental health disorders emerge in late childhood and early adolescence; not being attended to during that time increases the risks of more problems later in life (Das et al., 2016; Lehtimaki et al., 2020). The early emergence of psychopathology typically contributes to a lifelong trajectory of challenges across many functional domains (Lehtimaki et al., 2020).

According to the Centers for Disease Control and Prevention (2022) “Youth Risk Behavior Surveillance Data Summary & Trends Report,” 42% of students feel chronically sad or hopeless, with 29% experiencing poor mental health. The same report shows that 22% of

students seriously considered attempting suicide, and 10% have attempted it. As it pertains to gender, the Centers for Disease Control and Prevention (CDC) report found that 60% of female students experienced persistent feelings of sadness or hopelessness during the past year.

Interventions, however, have shown promise among the adolescent population base. In a systematic review of published literature, Das et al. (2016) found that evaluating mental health interventions among adolescents was very heterogeneous. Statistically, the outcomes varied in relation to the demographics and the individuals suffering from them. Variations came in efficacy for specific treatments and interventions among gender, age groups, socioeconomic status, and geographic settings, with the caveat that most interventions were superior to no interventions or treatment (Das et al., 2016). Schleider et al. (2020) concluded that something as simple as single session interventions provide a cost-effective and efficacious capacity to reduce and prevent youth psychopathology on multiple levels. With recent connectivity regarding telehealth, some proponents have touted online interventions as a means to reduce disparities related to adolescent demographic groups who do not have easy access to care. Lehtimaki et al. (2021) evaluated 18 systematic reviews and meta-analyses of such programs for adolescent mental health intervention. They found evidence of the efficacy of computerized cognitive behavioral therapy on anxiety and depression, with other digital media health interventions remaining inconclusive. As it stands, estimates suggest that 80% of youth with mental health needs go without services even though the United States spends more on mental health than any other nation on Earth (Schleider et al., 2020). This indicates that while programs that could help the youth exist, not all youths benefit from the possible benefits. One of the main reasons why these services are not available to all is the racial disparities that are a bedrock of the US society.

Children of Color, Social Media Use, and Mental Health

Today, children are born into a world full of technological gadgets. These gadgets necessitate social and digital media literacy. Digital media use in the United States has been on the rise (Twenge, 2020). As it stands, 94.6% of teens use social media, but only 3.5% reported using it to seek health-related information (Plaisime et al., 2020). For Black and Latino youth, estimates of social media usage stand at more than 70% (Tao & Fisher, 2021). Increases in depression among adolescents have been concurrent with the aforementioned increases in digital media use (Twenge, 2020; Weigle & Shafi, 2024). Twenge (2020) highlighted that the extent of this phenomenon is such that it can be identified as a mental health crisis, with girls and young women being the most impacted. From a historical perspective, national suicide and self-harm rates among teens and pre-teens were stable in 2010 (Estes, 2022). Since then, they have been consistently on the rise, thereby making mental health instability among these vulnerable groups closely correlated to rising trends in social media usage (Estes, 2022). For teenage girls specifically, Scully et al. (2023) found that body dissatisfaction was related to time spent engaged in social media comparisons and social comparisons with female targets while online. If a young girl evaluated themselves less favorably than the target group of close friends, this was more strongly associated with poorer body image appraisal (Scully et al., 2023). While body image comparisons are not unique to females, most of the literature selected for this study primarily characterized it as a female issue in terms of statistical relevance.

Race also affects how individuals navigate and respond to social media. Weser et al. (2021) concluded that Black teen girls uniquely navigate social media, and they are highly attuned to signs of bias. Jennings (2020) additionally stated that too little has been analyzed related to Black women's representation in digital spaces. In these spaces, women of color imagine gender performance, engage in participatory culture, and are exposed to hegemonic

elements (Jennings, 2020). Holistically, Black girls reflect on technology, and it informs the way in which they know and exist in the world to varying degrees (Garcia et al., 2020). The virtual world and the world outside the internet become conflicting dichotomies that impact each other. For example, the way in which Black girls have familial connections offline influences the content they produce on social media and their response to related content (Wade, 2019).

Black girls and women, historically and even presently, have faced a unique set of challenges in both offline and online spaces. These challenges stem from the intersectionality of their race and gender, often resulting in their experiences being marginalized and overlooked (Maragh-Lloyd, 2020). Historically, they have been subjected to harmful stereotypes and misrepresentations in popular media, perpetuating damaging narratives about their identity and worth. This legacy has translated into the digital world, where Black girls face disproportionate levels of online harassment, cyberbullying, and discrimination (Maragh-Lloyd, 2020). Without sounding repetitive, the prevalence of Eurocentric beauty standards and colorism in online spaces can contribute to body image issues and internalized racism. These intersecting challenges can significantly impact the mental health of Black girls, contributing to anxiety, depression, and a diminished sense of self-worth. This points to the urgent need for targeted interventions that address this population's specific experiences and needs, fostering resilience and empowering them to navigate the complexities of social media while preserving their mental well-being.

Popular media in all forms has long exacerbated stereotypical images of Black women (Jean et al., 2022). Evidence suggests that these images contribute to various negative psychological and behavioral consequences for Black women and girls that include but are not limited to problematic self-image, poorer relationships, and adverse mental health (Jean et al., 2022). This phenomenon has several implications for identity research and socialization among

Black girls and their development (Rogers et al., 2021). Beyond exposure to problematic stereotypical images, Del Toro and Wang (2023) highlighted that Black youth experience increases in online racial discrimination. This discrimination predicted problematic same-day and next-day mental health for Black youth but not for White youths (Del Toro & Wang, 2023). This builds on Weser et al.'s (2021) notion that Black female teens navigate and are impacted by social media differently, and it extends this to Black teens in general.

Researchers Mady et al. (2023) contended that skin color affects women's self-image, and this has long been consequential of racial discrimination and the presence of light-skinned models in modern advertisements and forms of communication. Though not peer-reviewed, The Fashion Spot conducts studies that analyze representations of race in the fashion world annually to track progress in inclusivity within the fashion industry. According to Schimminger (2022), their study found that 78% of models in fashion advertisements are White, and only 22% are Black, Asian or Hispanic. For young women, Choukas-Bradley et al. (2022) found that body image concerns were a key mechanism between adolescent girls' social media use and mental health. In this study, when young girls focus on idealized images of beauty as propagated through peers, influencers, celebrities, and models, it causes them anxiety about their appearance (Choukas-Bradley et al., 2022). When those images are primarily a skin color different than their own, this creates a problematic paradigm for emulation. Saraswati (2020) highlighted this as a systemic process in popular media images that pushes girls of color to attempt to emulate Whiteness and upper-class White populations. Media is critical in producing sociological pressures (Santoniccolo et al., 2023). Beyond color divides, there are also gender divides. Santoniccolo et al. (2023) found that gender in media representations typically features gender stereotypes, objectification of women, and sexualization of women, which in turn have been shown to correlate with detrimental effects on the physical and well-being of women. Patterns of

eating disorder, increased body awareness and poor body image have all been correlated with the phenomenon (Santoniccolo et al., 2023). For women of color, they have double jeopardy in terms of negative media depictions based on gender and race.

Online racial discrimination has implications for impacting mental health disparities that already disadvantage black youths (Del Toro & Wang, 2023). Tao and Fisher (2021) concluded that racial discrimination on social media platforms for adolescents of color led to increased anxiety, depression, and alcohol use. Wade (2019) determined that the unique way adolescents of color experience social media can also positively impact mental health. The researcher showed that Black girls could form support networks online to control spaces often denied to them in their everyday lives and school environments. Instructing or training them in how to do so would be necessary for the potential of this to be reached. Overall and specifically for Black teen girls, focus groups have shown that their experiences on social media included racism, biases, being associated with Black stereotypes, cultural invalidation, and being accused of acting White (Weser et al., 2021). While the amalgamation of these factors can have disastrous outcomes for Black and Brown female teens and pre-teens, unexpected issues such as COVID-19 exacerbate the risks, making the lives of these young people online hell on earth.

COVID-19 and Its Impact on Social Issues

While each population was affected by the COVID-19 pandemic in some way, the consequences of the pandemic were felt differently in relation to individuals' status and the demographics in which they reflected (Reyes, 2020). Marciano et al. (2022) highlighted that, in general, the COVID-19 distancing dynamic had detrimental effects on adolescent mental health. According to research by Reyes (2020), minorities were disproportionately impacted by the pandemic, with African Americans having lower support, higher mortality, greater financial burden, and more problematic social determinants of health in general. Beyond minority status,

there has been some evidence to suggest that young people are vulnerable to the mental health impacts of COVID-19. Due to traditional social constraints, this period also saw a rise in social media use. Bailey et al. (2022) found that young people during COVID-19 reported high levels of psychological distress, with over 40% of those individuals reporting severe levels of anxiety and depression. These findings were more severe when individuals had preexisting mental health issues or were from gender-diverse populations. The researchers continued and found that across all young participants, social media use was high, with 96% using it at least once a day and two thirds stating their usage levels increased after the pandemic.

Bozzola et al. (2022) reported that during the pandemic, internet use allowed users to communicate with peers and continue school-related education activities. The researchers further stated that social media services at this time enabled internet-based networking, brought people together, and facilitated instant connections in a time of isolation. Connecting the race/age themes, Banks (2022) found that Black adolescents are the most vulnerable to COVID-19, with high mental health service needs and low service utilization. One of the most common blocks to mental health treatment during this time was financial issues (Banks, 2022). It was not uncommon during COVID-19 for young people to seek support for suicidal thoughts or self-harm through social media or to use social media to support another young person in crisis (Bailey et al., 2022).

Research Exploring Mental Health Services

This section examines the research and findings related to current mental health services offered by the government and non-profit organizations to Black and minority communities. It also highlights the challenges in delivering these mental health services, especially concerning the negative impact of social media on the mental health of Black and minority youth.

Racial Disparities in Access to Mental Health Services

The degree to which racial health equity in the United States is a social problem has been heavily documented by diverse stakeholders. The Secretary's Task Force on Black and Minority Health highlighted that there was a *national paradox* between Black-White health disparities over three decades ago (Neighbors et al., 2023). According to research conducted by Neighbors et al. (2023), Black Americans continue to show poorer health in comparison to White Americans, with existing problems present on the psychological, behavioral, social, and economic levels. This can be illustrated in several ways. One way would be within the context of the obesity epidemic. African American women, though only 13% of the population, have the highest rates of obesity in the country and are more likely to die at early ages (Abedi et al., 2021). As it pertains specifically to mental health, Duncan (2023) found that African Americans were 20% more likely to experience serious psychological distress compared to White Americans, with Black women being about half as likely to seek mental health care as their counterparts. In Black female high school students, disparity-related statistics highlighted that this group is 60% more likely to attempt suicide than non-Hispanic White females of the same age (Duncan, 2023).

In the previous section, with the negative elements associated with social media use in research, this facet would provide a strong foundation and lens for further exploration of how race interacts with the harmful elements of social media usage. This is true because social determinants of health are factors that are related to the conditions in which individuals live, learn, work, and conduct leisure activities (CDC, 2023). The convergence of these factors - systemic health inequities, the negative mental health impact of social media, and the unique vulnerabilities of BBFTPTs- creates a pressing need for targeted interventions. Understanding

the specific needs and challenges of BBFTPTs in the context of social media use is critical as it helps with working towards more equitable and effective mental health support systems.

Government and Private Services for Social Media Impact

While social media's impact on young people is not fully understood and understudied, this does not mean that various stakeholders do not know the related risk factors in the public and private sectors. According to the United States Department of Health and Human Services ([HHS),2024), social meaning presents harmful risks to youth, but it also has the potential to provide benefits. This is backed by HHS (2024) statistical data that stated children and adolescents who spend more than 3 hours a day on social media are 2 times more at risk for mental health problems than children who use it less. The same study shows that teenagers spend an average of 3.5 hours on social media daily, illustrating the breadth of this problem. Murthy (2024), the United States Surgeon General, recently made the decree that there is a mental health crisis among young people in the country, and social media is an important contributor to this. Though he says that the nation does not have the luxury of perfect information on the topic, based on assessing the available facts, judgement warrants quick action (Murthy, 2024). This is further substantiated by the notion that 46% of adolescents aged 13-17 said that social media makes them feel worse about their bodies (HHS, 2024; Murthy, 2024). The White House (2024) has also reinforced these notions. President Biden stated that he is committed to making the youth mental health crisis a top priority and would support legislation strengthening children’s privacy, health, and safety online. While this is somewhat ambiguous, Murthy (2024) has officially called for issuing warning labels on social media platforms while more data can be accessed.

This overview of government and private sector perspectives on the impact of social media on young people, particularly their mental health, emphasizes the urgency and relevance

of this research. While there is growing recognition of the potential harms of social media, especially for vulnerable populations like BBFTPTs, there is a lack of targeted interventions and policies addressing these specific challenges. This research seeks to bridge this gap by examining the effectiveness of existing mental health training programs designed for this demographic and identifying key factors that influence their implementation. By shedding light on these issues, this study aims to contribute to the development of evidence-based strategies and policies that can better protect and support the mental well-being of young people in the digital age.

Mental Health Training to Address Issues Caused by Social Media

As the previous section highlighted, various governmental and organizational stakeholders have studied social media's impact on adolescents and organized training for mental health professionals related to the phenomenon. The efficacy of training, however, is harder to substantiate, and which interventions are most suitable for which groups is still a matter in which there is as much known as there is unknown. The wisdom behind mental health training to help professionals effectively support adolescents dealing with social media-related mental health difficulties is rooted in recognition/response (awareness), evidence-based strategies, enhanced communication skills, and holistic support systems. According to O'Reilly (2020), practitioners must be able to assess risk, and social media is a new dimension that they need to consider beyond the traditional risk elements. Within this paradigm, however, the researcher contends that there is a propensity for mental health professionals to immediately equate social media with negative rhetoric about adolescents using it. For success, it is necessary for mental health practitioners to consider social media when risk-assessing adolescents without assuming that the relationship is always negative (O'Reilly, 2020). A degree of objectivity is required, and this would have to be echoed in training protocols.

Electronic device and social media use correlatives to health risks is an understudied phenomenon, according to Estes (2022). This is of small wonder in that social media is still in its infancy as a mainstay of modern human culture. What has been known for some time, however, is that electronic devices have addictive properties (Estes, 2022). The health risks associated with social media are not well-known among mental health providers. However, research conducted by Estes (2022) highlighted that when proper training based on awareness of social media/health risks was provided to caregivers, they were more likely to change their own digital device usage and recommend patients also to do so. This suggests that greater awareness can lead to change, and mental health training to address issues caused by social media is, first and foremost, awareness based. Bozzola et al. (2022) stated that families should be educating their children on the dangers of children and adolescents being online. Many of those parents, however, are also not aware of the full scope of risks associated with young people and internet usage. To counter this, Bozzola et al. suggested that health practitioners be educated and trained so they can inform families how to handle social media use. The training suggested by the researchers would include knowledge of the correlation to negative social media use elements like depression, obesity, psychological problems, sleeping issues, addiction, sex-related problems, body image problems, physical inactivity, online grooming, and anxiety.

The type of dynamic training suggested by Bozzola et al. (2022) is backed by research. However, other data shows that training should be multifaceted and extended beyond just training mental health professionals. For example, Del Toro and Wang (2023) suggested that programs can be implemented to decrease online hate crimes and health providers should be developing procedures to mitigate negative mental health effects following online racial discrimination experiences. Tam et al. (2024) took the perspective of media induced mental health awareness campaigns and did a robust meta-analysis of studies using such programs

published from 2004 to 2022. According to the researchers, mental health awareness campaigns through mass media were associated with positive changes in the attitudes, beliefs, and intensions of young people and positive alterations in behaviors for help seeking. A single modality to addressing problems with mental health and social media use, therefore, would be inferior to programs, education, and campaigns that take multimodal perspectives on the subject. The reason is that mental health, race, adolescence, and social media are all extremely complex subjects that can interact with one another in varying capacities.

ATBS, A Triad Company, offers a three-credit course on social media and mental health that is geared toward LMFTs, social workers, LPCCs, nurses, substance abuse counselors, and other mental health clinicians. The course is built on the theoretical underpinning that the ability to stay constantly connected can impact a person's mental health and well-being (AATBS, 2024). The program focuses on awareness related to internet addiction and social media addiction, with attention to both the negative and potential benefits of the phenomena (AATBS, 2024; Sun & Zhang, 2021). Similarly, the eCare Behavioral Health Institute (2024), shortened to eCare, offers a course on the impact of social media on adolescents with the intent of guiding mental health professionals. The course claims to show professionals how they can counteract the mental health effects of social media in teenage clients using CBT and DBT skills using evidence-based data (eCare, 2024). Topics examined in the training are cognitive restructuring, mindfulness, distress tolerance, and emotional regulation (eCare, 2024). The wisdom behind these trainings is that professionals can help young people develop better skills for reducing social media's harm and increasing its positive elements.

Beyond training for mental health professionals, there is also a movement to train young people to use social media in a proactive way. According to the APA's recommendations for adolescent media use (2023), just as young people are required to get training to get a driver's

license, they need instruction in the safe and healthy use of social media. This notion is based on evidence. As highlighted by Hernández-Martín et al. (2021), pre-adolescents (11-13) who use social media more often than their peers do not display better digital companies in the areas of safety related to knowledge, skills, and attitudes. Without better skill sets but with more time spent with social media, these pre-adolescents would be more at risk than their peers based on the exposure amount. The APA (2023) continued suggesting that training and interventions for young people related to social media use should be age-appropriate, should be based on their level of maturity, and should focus on self-regulation skills, intellectual development, and comprehension of risks. The interventions would focus on increasing adolescent skill sets for critically evaluating social media. There are other organizations that offer programs geared toward children and social media use. GKIS (2024) offers self-paced social media readiness courses in 10 lessons with mastery quizzes that also come with free parent versions for each source. The organization suggests that their training prepares tweens and teens for safer screen use to prevent psychological illness through emotional wellness tools (GKIS, 2024). While GKIS (2024) presents evidence for their program's efficacy, there is a lack of external evaluation data specifically pertaining to their courses to determine with certainty the degree to which they are effective.

Since the degree to which social media negatively impacts young people differs from one individual to the next, this would suggest that there are certain skills or support that certain young people possess that make them more resilient. It would be logical for training interventions to focus on these skill sets. Unfortunately, there is a considerable lack of data related to which techniques and which skill sets will keep young people safe while using social media. Two skills that have been proposed beyond simply having a better awareness of the negative impact of social media include emotional self-regulation and self-efficacy. In study

conducted during the pandemic, a time of increased social media use, Shao et al. (2021) found that emotional self-regulation helped individuals counteract the negative impacts of social media. Additionally, Shao et al. (2021) found that social media also challenged and made a person's emotional self-regulation less efficacious.

For self-efficacy, Faelens et al. (2021) found that using Facebook and Instagram lowered self-esteem and emotional regulation. This would suggest that a person who began using the platforms with higher self-esteem and emotional regulation levels would have fewer problematic consequences than those who began with lower baselines. Intrapersonal self-regulation, which includes cognitive reappraisal, mindfulness, interpersonal level supportive interaction, and self-kindness, is all evidence-based to buffer emotional exhaustion related to a specific situation (Shao et al., 2021). Social media, when perceived in its negative context, could be the type of specific stress-inducing catalyst from which interpersonal self-regulation could be beneficial. Professionals could be trained to help develop self-efficacy and emotional regulation in young people to equip them better to deal with social media trappings after the young people are made aware of the potential trappings.

While awareness-based training for mental health professionals and educators has shown promise in mitigating the negative impacts of social media on adolescents, a multi-faceted approach encompassing programs, education, and campaigns is essential. This echoes the principles of social cognitive theory, highlighting the importance of not only addressing individual coping mechanisms but also the environmental and social influences that shape young people's online experiences. By fostering skills such as emotional self-regulation and self-efficacy, training programs can help empower young people to navigate social media more critically and resist its negative influences. This approach also aligns with the recognition of racism within online environments, suggesting a need for interventions that equip BBFTPTs

with the tools to navigate both the psychological challenges of social media and the systemic biases they encounter online. Therefore, effective mental health training should go beyond simple awareness, focusing on building resilience, fostering critical media literacy skills, and promoting a positive online environment. This multipronged approach will ultimately empower young people to leverage the benefits of social media while mitigating its potential harms, paving the way for healthier and more fulfilling online interactions.

Non-Profit Organizations' Design and Delivery of Services

Non-profit organizations are instrumental in delivering tailored mental health services, particularly to vulnerable populations such as BBFTPTs. These organizations carefully design and implement programs that address the unique challenges faced by these groups, especially concerning the negative impacts of social media on mental health. The design of these services is grounded in cultural competence and trauma-informed care, ensuring that the programs resonate with their target audience's specific needs and experiences (Castillo, 2020). Mental health training programs offered by these organizations focus on building resilience, teaching effective coping strategies, and fostering positive self-esteem. These programs often include workshops or group therapy sessions where participants can learn about healthy digital habits, critical thinking, and self-care techniques. Furthermore, non-profits create community-based support systems, establishing safe spaces where young girls can discuss their experiences and receive emotional and psychological support. In many instances, these organizations also collaborate with schools to integrate mental health education into the curriculum, developing specialized modules that address the impact of social media on the mental health of BBFTPTs.

Information Collection and Service Review

Nonprofits are committed to the continuous improvement of their services through diligent information collection and service review processes. These organizations actively gather

participant feedback, monitor mental health outcomes, and stay abreast of the latest research on social media's impact on adolescent mental health. The data collected is used to refine and enhance the services provided, ensuring they meet the evolving needs of the communities served. As an example, participant feedback or new findings from mental health studies may lead to adjustments in program content or delivery methods. This ongoing process of review and adaptation is crucial for maintaining the relevance and effectiveness of the services offered by non-profits.

Pressures and Limitations

Nonprofit organizations face significant pressures to act, driven by the growing mental health crisis among adolescents, particularly within minority groups. The urgency of their work is underscored by the high rates of depression, anxiety, and suicide among BBFTPTs, exacerbated by the harmful effects of social media, as illustrated in earlier sections. Despite their critical role, these organizations often encounter substantial limitations. Funding constraints are a persistent challenge, limiting the scope and scale of their programs and restricting their ability to reach a broader population or offer more comprehensive services. In addition, non-profits frequently operate within systems that do not fully support their efforts. For instance, the absence of a standardized framework for addressing the mental health impacts of social media in educational settings can hinder the integration of their programs into schools. Resource limitations, such as a lack of trained mental health professionals or access to the latest technology, further impede their ability to offer cutting-edge interventions (Kadariya et al., 2023). These pressures and limitations underscore the need for more robust support systems and innovative strategies to enhance the effectiveness of non-profit interventions.

Theoretical Framework

This section offers a comprehensive overview of the theoretical foundations established by three significant theories: change management theory, institutional theory, and resource dependency theory. Furthermore, it delineates the relevance of these theoretical frameworks in addressing the research question and identifying effective strategies for mitigating the identified problem.

Change Management Theory

The development of a curriculum aimed at helping BBFTPTs avoid the negative consequences of social media requires a robust theoretical framework to guide the change within NPOs. Methods for change management can be traced back to early human history (Ratana et al., 2020). Despite widespread discourse and understanding the need for change is central for survival, most organizational endeavors at change management ultimately fail. According to the work of Errida and Lotfi (2021), the estimated failure rate of organizational change initiatives is as high as 60-70%. This is due to a variety of factors. Looking from an overarching perspective, Ratana et al. (2020) established that achieving effective change requires more than a set formula; instead, it demands a comprehensive integration of the various factors essential to the change process.

The first selected theoretical framework for the study, change management theory, is rooted in the work of Kurt Lewin, who developed a change management model in the 1940's from which all subsequent change management theories are connected to with varying degrees. Lewin's three-part change management style is a process that unfreezes the problematic behavior, makes the change through empowerment and addressing barriers to resistance, and then ultimately refreezing the change (Crosby, 2020). Some critiques of this perspective suggest that it is an oversimplification of a complex phenomenon; however, Burnes (2020) highlighted

that it is still an evidence-based approach to change based on organizational management. Change management theory helps people and organizations focus on the future and make the right decisions to get to that vision. In this case, the change management theory emphasizes the need for nonprofit organizations to adapt and initiate change, allowing them to respond effectively to the challenges posed by social media.

As identified as the salient problem that needs addressed, Illinois nonprofits have stepped up to provide mental health training programs explicitly targeted BBFTPTs; however, none of these programs explicitly deal with the complexities of social media targeted at these youth. Additionally, they offer no specific targeted methods to mitigate its negative impact on their mental health. The initial step for unfreezing requires recognition of the pressing need for the change. The present methods and training address mental health but not the profound and unique challenges posed by social media to the respective group. To unfreeze the current system, nonprofit organizations must acknowledge this missing component and create an understanding among stakeholders that a new, specialized approach is essential. This could be accomplished through awareness campaigns, discussions, and preliminary workshops to build a foundation for the shift in focus.

The change portion starts after the groundwork has been laid. Nonprofits can start by incorporating evidence-based specialized content that addresses social media's impact on the targeted BBFTPTs. This would include developing targeted coping techniques, promoting digital literacy, and guiding teens on managing their online presence. In collaboration with mental health professionals, program developers can design new modules or activities to help these youth critically analyze social media content, foster resilience, and recognize both the positive and negative implications of online interactions. During this phase, it is critical to

support the staff and participants through training, resources, and a clear focus on the program's goals (Burnes, 2020; Crosby, 2020).

The final phase, refreezing, would occur after these updated program elements have been integrated. This would involve regularly evaluating the impact of the new modules, gathering feedback from participants, and refining approaches based on their experiences. Rather than a destination, refreezing is still a process where continuous improvement can manifest. Ongoing support should be ensured for program facilitators to keep them equipped with the latest insights into social media's evolving trends and challenges. By embedding these practices into the nonprofit's standard program offerings, the organization can help normalized conversations around social media's impact, making it a permanent and effective part of mental health support for BBFTPTs.

Institutional Theory

Beyond change management theory, the institutional theory will also be used. The institutional theory highlights the necessity of dismantling outdated structures and overcoming both internal and external barriers to achieve meaningful change. Old structures need to shift and fall, change must happen from within and without, and this necessitates removal of barriers and supporting NPOs in developing curriculum to help BBFTPTs avoid the negative consequences of social media. Institutional theory was popularized by sociologists and organizational theorists like Paul J. DiMaggio and Walter W. Powell. The two scholars published a 1983 paper called, "The Iron Cage Revisited; Institutional Isomorphism and Collective Rationality in Organizational Fields." As a cornerstone of institutional theory, their work expanded on Max Weber's ideas and introduced the concept of institutional isomorphism, which highlights how organizations in a field often become similar over time due to pressures to confirm to norms, values, and structures (DiMaggio & Powell, 1983). This process can limit

potential efficacy as if there were already a weakness in place or a societal shift has occurred, the echo chamber homogenization process would make it worse.

The current mental health programs, while well-intentioned, operate within an institutional field that reflects established norms, resources, and practices. These programs are rooted in a traditional approach to mental health training that does not fully consider the evolving dynamics of social media's impact on young, especially within marginalized communities or within the unique perspectives of female, BBFTPTs. According to institutional theory, to achieve meaningful and sustained changes, nonprofits must go beyond surface-level adjustments and focus on dismantling and reshaping these outdates program structures and assumptions.

To address the unique mental health challenges on social media to the targeted group, nonprofits can begin by recognizing how internal barriers, such as rigid program structures and limited perspectives contribute to the gap. Many organizations may resist change due to pressures to adhere to proven models or due to resource constraints, which reflects what DiMaggio and Powell (1983) references as institutional isomorphism. These nonprofits can create an effective, targeted response by innovating to break away from these conventions. Externally, nonprofits also face pressures from funders, stakeholders, and regulatory bodies, which may favor traditional program outcomes over experimental, social media-focused mental health initiatives. Overcoming these external barriers requires advocacy and collaboration with other stakeholders to shift perception about what constitutes an effective mental health intervention for this demographic.

Resource Dependency Theory

Resource dependency theory (RDT) is a framework that was developed by Jeffrey Pfeffer and Gerald R. Salancik in the 1970s. The theory contends that entities controlling critical

resources can exert power over an organization, thereby influencing its decisions and actions (Pfeffer & Salanick, 2015). In this capacity, an organization's survival is contingent on its ability to acquire and maintain resources (Pfeffer & Salanick, 2015). Nonprofits rely heavily on external funding from grants, donors, and government sources. Prentice and Clerking (2023) illustrated that several financing and operational variables impact donations. Funders often dictate program priorities thereby limiting nonprofits' flexibility to focus on social media-specific health issues.

In total, Illinois nonprofits face a dependency on specialized expertise and partnerships to effectively address social media's mental health impacts on BBFTPTs. They rely on external organizations, like mental health researchers and tech companies, for the knowledge and resources needed to tailor their programs. This makes it challenging to address complex social media issues independently. Furthermore, as in the case of institutional theory, mental health programs operate within an institutional field that reflects norms for programing, resources, and practices. Moving beyond this can be difficult. This reflects Pfeffer and Salanick's (2015) assertion that organizations are inescapably bound up with conditions of their environment. Nonprofits are going to have to navigate the power dynamics present in their field.

This review demonstrates that combining change management theory, institutional theory, and resource dependency theory provides a comprehensive framework for developing an effective curriculum to support the mental health of BBFTPTs facing the unique pressures of social media. While change management theory offers a structured, step-by-step approach to implementing new practices, institutional theory adds a deeper dimension by addressing the systemic and cultural barriers inherent in traditional nonprofit structures. RDT illustrates how nonprofits rely on external resources like funding, expertise, and partnerships to create and

sustain targeted mental health programs. Together, these theories highlight the need for both internal adaptation and external support to dismantle outdated practices and build a more resilient and efficacious program structure. This combined approach ensures that the nonprofits can innovate to meet the distinct needs of this population while fostering resource-sustainable, organization-wide transformation that aligns with evolving social media challenges.

Literature Synthesis

Based on the research gathered, there are few key take-aways related to mental health, training, and the influence of social media on female tens and pre-teens of color. The first is that social media use is on the rise and social media has been proven through literature to have negative impacts on mental health. These problems grew during the COVID-19 pandemic. Not all elements of social media use are negative; however, young people are more susceptible to the trappings of social media. Of young people, certain groups are more vulnerable based on their skill sets, maturity, the way in which they view themselves, their support structures, and their environment. Female teens and pre-teens of color experience social media in a unique way. They are particularly sensitive to the racism that is ingrained in online environments (Volpe et al., 2021). Additionally, since learning manifests through social contexts per Bandura (1986), social media is a virtual environment that can influence targeted groups in either positive or negative ways.

The government is aware of the dangers of social media to young people to the extent that they have declared there is a mental health crisis facing the nation, with social media being a key contributor. To combat this issue, mental health training has shown promise for helping professionals address the issues caused by social media in young people and to help young people develop skills and awareness to make them more resilient to the harmful effects of social media. Non-profit organizations provide culturally competent mental health services to

BBFTPTs, focusing on resilience-building and coping strategies to counter the negative impacts of social media. They continuously gather feedback and adapt their programs to better meet the needs of their communities, often working in collaboration with schools. However, these organizations face significant challenges, including limited funding, systemic barriers, and resource constraints, which impact the scope and effectiveness of their interventions.

Despite the substantial knowledge on the harmful mental health effects of social media among adolescents available in the literature, there still exists critical knowledge gaps regarding the appropriate approach to these democratizing challenges among BBFTPTs. Although there is previous research confirming that, compared to general populations, BBFTPTs are particularly vulnerable to online racial discrimination and exposure to sexist ideals, there is little research focusing on how, where, and why mental health training programs should target the social media-related needs of this population. The existing nonprofit interventions do not critically assess their culturally resonant strategies and evidence-based practices to minimize the negative impact of social media on BBFTPTs. This study filled this gap by exploring how nonprofits in Illinois can design and use mental health training courses to enable BBFTPTs to not only decode the psychological threats of social media but also cultivate digital literacy and resilience.

Chapter Summary

This extensive literature review successfully forms the necessary basis for reviewing the intricate correlation between social media use and the mental health of BBFTPTs. The chapter also effectively shows how the historical development of social media, which is further enhanced by the isolation processes because of the COVID-19 pandemic, puts unprecedented pressure on this vulnerable group that is discriminated against in an intersectional manner. The systematic organization of four main sections explores the current research in the field, but identifies the notable deficit in culturally competent interventions. The combination of change

management theory, institutional theory, and resource dependency theory provides a strong theoretical spectrum with which to consider the organizations as well as the external challenges to developing targeted mental health programs by nonprofits. The next chapter encompasses the qualitative research methodology, research design, selection of participants, and data collection process to determine how Illinois nonprofits can incorporate social media-driven content into their mental health training programs of BBFTPTs.

CHAPTER III: METHODOLOGY

This chapter introduces the qualitative methodology, research design, sample selection, research site, data analysis approach, and ethical considerations that informed the study. An explanation of the processes establishes knowledge for readers to understand how each step took place and how it could contribute to the overall research. This foundation also provides transparency and rigor, allowing readers to assess the credibility and trustworthiness of the study's findings.

Research Design

The qualitative study used an exploratory design to raise understanding about the integration of the negative influence of social media topics in curricula. This methodology and design were appropriate for the proposed research question because it allowed us to take an in-depth look into a topic that is understudied (Makri & Neely, 2021). Qualitative studies often have complex phenomena that require in-depth understanding by exploring the facts presented and the data collected. The exploratory design approach helps researchers investigate complexities in a topic as they look to interpret data for their readers (Alhazmi & Kaufmann, 2022). Additionally, the exploratory approach offers flexibility and adaptability, where researchers can adjust their questions to focus on new insights that crop up during a study. Alhazmi and Kaufmann (2022) highlighted that exploratory research ensures researchers have a sense of flexibility as they navigate the different complexities they face during research. Seeing as it is a "why research," this design is effective as it will help me engage freely with participants, allowing me to identify patterns from the data given.

Research Participants

Participants who are actively involved with non-profits in Illinois that deal with the mental health assessments of the population were used. Negrin et al. (2022) highlighted that

participant recruitment is the basis of every qualitative study since it provides a foundation for the methodology. The study aimed to collect data from ten participants, five non-profit leaders, and five mental health professionals. In the end, 8 participants were surveyed. According to Yin (2018), 7-10 participants are sufficient for obtaining an appropriate depth of data saturation in a study, meaning that participants will begin to share the same information rather than anything new. These participants were selected based on their expertise and experience in mental health services to the population in Illinois. Each has played, or continues to play, a critical role in delivering mental health services to the affected population in Illinois, thereby facilitating access to essential care that promotes improved mental health outcomes.

The participants were asked to share their opinions and experiences regarding the implementation of the Illinois mental health training programs and whether they address the negative influences of social media issues, particularly on BBFTPTs. The chosen purposive sample brought together professionals from core fields in the mental health field who have taken an active role in dealing with mental health issues in Illinois and ensuring there were desirable outcomes in the short and long term. Prosek and Gibson (2021) support using participants relevant to a study since they provide primary data that gives more information about a topic under consideration. Therefore, the selected sample provided diverse perspectives, enabling the generation of rich data to explore the research question.

Sampling Technique

In research, accuracy is vital to building a strong case based on the topic under discussion. Sampling involves selecting participants from a larger population to be part of the research process. When choosing an effective sampling technique, researchers consider accuracy so that the results they present reflect the actual situation of the topic under study. Campbell et al. (2020) highlighted that purposive sampling allows researchers to select participants who best

contribute to the research process. Since the research involved understanding the nuanced ways BBFTPTs get support from mental health training programs from non-profits in Illinois, it was vital to have participants who provided in-depth information. The approach aligned with the study's aim to understand how the Illinois mental health training program assists BBFTPTs in coping with the negative influences of social media.

Purposive sampling also allowed for the elimination of bias during the interview process since the researcher collected information from multiple participants. Cash et al. (2022) highlighted that any mismatch between the proposed sample and the population under study can result in bias. Therefore, purposive sampling ensured the selected sample represented the entire population in the study to eliminate bias that could have rendered the entire research invalid. Purposive sampling was effective for this research study since it allowed me to narrow down the participants to nonprofit leaders and mental health professionals in Illinois. The participants had in-depth information on the current mental health status of BBFTPTs in Illinois. The researcher also selected individuals who were able to provide insight into how various factors influenced the integration of educational training curricula for Illinois nonprofits addressing the negative impact of social media on BBFTPTs.

Data Collection Procedure

During data collection, researchers employ varied approaches that ensure accuracy and effectiveness throughout the process and during analysis. However, researchers should select the right data collection process that fits into the research process and topic under consideration (Mwita, 2022). Using the wrong data collection approach compromises the research process and limits the chances of success and eligibility of the research. According to Collins (2021), researchers can use instruments to track their records and store essential information that is vital to answering research questions. In qualitative research, interviews are the most commonly used

instruments in data collection. They involve direct and indirect contact with participants to collect essential information needed to understand factors influencing a subject.

This qualitative study employed a primary data collection instrument, conducting interviews with all participants selected for the research. Using the right instrument ensures that researchers collect information that makes the research process and data collected impactful. To facilitate a flexible yet guided conversation with research participants, the interview questions were designed in a semi-structured format. This approach was selected because it enabled the capture of rich, qualitative data that explored the complexities of the research topic, complementing the structured data collected through surveys and program analyses (Ruslin et al., 2022). Literature suggests that semi-structured interviews ensure researchers have an open-ended interaction with participants to gather as much information as they can (Mwita, 2022). By selecting this approach, the study ensured that key themes were thoroughly explored while also allowing for the emergence of unexpected insights and the opportunity for participants to elaborate on their individual experiences.

The selection of agencies for participant recruitment was guided by specific criteria designed to ensure the relevance and richness of the collected data. The first criterion was that (1) the agencies had to have been Illinois-based nonprofits actively providing mental health training programs specifically targeting preteen and teenage BBFTPTs. The second criterion was that (2) the agencies' mental health training programs had at least discussed offering services to support those suffering from the negative impacts of social media. This guaranteed that the participants could offer relevant perspectives on the research question. The third criterion was that (3) the agencies demonstrated a track record of strong community engagement and cultural competency in their work with BBFTPTs. Importantly, the agency had to have had some form of program evaluation data available, even if it was only for internal use.

This study aimed to enhance understanding of how various factors influence the integration of educational training curricula for Illinois nonprofit organizations regarding the negative impact of social media on Black, Brown, and transgender people (BBFTPTs). There is a lack of clarity about how mental health training programs in Illinois assist BBFTPTs who are facing mental health challenges due to social media. To clarify the role that nonprofits in Illinois play in addressing these issues, the study sought to determine the existence of a curriculum or the intention to develop one for assisting BBFTPTs. Through a series of interviews, the study gathered detailed information on whether BBFTPTs are receiving adequate support for mental health issues linked to social media. It also examined whether there is an existing curriculum for mental health training programs or plans to create one aimed at helping BBFTPTs cope with their mental health challenges. The study ensured that all interview questions focused on the core issues to provide valuable insights for readers and nonprofit organizations alike. The interviewed nonprofit leaders included program directors, mental health clinicians, curriculum developers, and community outreach coordinators.

Specifically, data collection for this research involved a two-pronged approach, utilizing semi-structured interviews and program analysis. Simultaneously, a comprehensive analysis of program curricula, educational materials, and evaluation data (if available) was conducted. The data was collected, transcribed, coded, and analyzed using thematic analysis to identify patterns and draw conclusions about the effectiveness of mental health training programs and the factors influencing their success in mitigating the negative impacts of social media on this vulnerable population.

The decision not to directly interview the BBFTPTs themselves was primarily based on ethical considerations. This population is particularly vulnerable, and direct questioning about their mental health and social media experiences could potentially cause distress or harm.

Furthermore, gaining informed consent from minors and their guardians can be a complex and time-consuming process. However, the perspectives of these BBFTPTs were indirectly captured through the interviews with the professionals who work closely with them. This approach allows for insights into their experiences while prioritizing their safety and well-being.

According to Mwita (2022), researchers should be accurate when collecting data to ensure they achieve high accuracy levels and eliminate any errors that impede the research process. Researchers should select the right approaches to make the research reliable and valid. Interviews are the most effective way to collect the vital data needed for the study and to build a testimony based on the topic. The semi-structured approach ensured that the participants were free to share information and thoughts based on the questions asked. See Appendix A for all interview questions.

The interviews were conducted via Zoom, recorded, and transcribed to ensure that the information and data shared were clear to all readers. Recording and transcribing the interviews was also vital to ensure transparency in the process and allowed for greater coding and analysis. All the participants were informed of the presence of recording devices before the interview commenced to ensure there was trust throughout the entire interview process

Data Analysis

Data analysis is a core part of the research process since it allows researchers to assess and interpret the data they collect to derive meaning for potential readers. According to Mattimoe et al. (2021), the data analysis phase in qualitative research is intellectually challenging as researchers have to actively interpret the data and themes they identify in collected data. Coding is a vital data analysis technique that allows researchers to label important features in the data they collect, which are relevant to the research question (Bryne, 2021). It ensures researchers can move from raw data to findings, where they can make sense of

the observations made. Coates et al. (2021) defined a code as a short representation that captures vital features of a main idea presented by subjects. Through coding, researchers can mark any features they identify during data analysis to help interpret themes effectively.

Coding was essential for this qualitative study as it enabled the researcher to capture key features while evaluating the collected data. The researcher utilized Microsoft Excel to code the data, which allowed for the classification of data sets and accurate interpretation of the identified themes. During the data analysis, the Braun and Clarke approach was employed to analyze patterns within the data. According to Byrne (2021), this approach allows researchers to incorporate creativity, subjectivity, and flexibility in producing insights from the data they collect. It facilitated the generation of initial codes as the researcher identified crucial features for data interpretation. For instance, the researcher coded content specifically related to the mental health impact of social media on BBFTPTs. This approach also helped establish a link between the effects of the curriculum on mental health training programs and the development of coping mechanisms among BBFTPTs facing mental health challenges due to social media.

Reliability and Validity

Researchers engage in qualitative research as they look to get deeper insight into social phenomena taking place within a specific area (Creswell & Poth, 2018). However, questions over validity and reliability often arise due to the subjective nature of qualitative data. Scott et al. (2019) highlighted that validity and reliability in qualitative research in making results impactful and reliable to the target audience. To make sure the research was valid and reliable, the researcher ensured all interview questions and data collected aligned with the research question pertaining to understanding how factors influence the integration of educational training curriculum for IL nonprofits about the negative influence of social media on BBFTPTs Both interview questions and data analyzed were sent to my committee to assess their appropriateness

and accuracy, to which they signed off on the material. Other techniques that were employed to achieve greater reliability and validity included triangulation, member checking, and maintaining a detailed audit trail to enhance the credibility, transferability, dependability, and confirmability of the findings (Creswell & Poth, 2018).

Researcher Role

As the founder and Executive Director of Focus Fairies, a Chicago-based nonprofit mentoring program for inner-city girls of color established in 2017, I brought both personal and professional experience to this research. My journey began with a desire to become the role model I lacked during my formative years, driving me to help youth from underprivileged circumstances overcome hardships similar to those I faced. I am currently directing Darra Michelle Events & Consulting, and I am pursuing an educational doctorate at Governors State University, focusing on mental health and community engagement. I have a Bachelor of Public Health, majoring in Healthcare Administration from Northern Illinois University & Master of Business Administration from Argosy University. Having worked professionally in multiple fields, I have developed ways of managing relations and projects in an environment that is constantly changing, enabling me to analyze this issue with impartiality and from an insider's view of the issues affecting BBFTPTs.

Ethical Considerations

Ethics and ethical guidelines are essential in research, as they help ensure that researchers adhere to established standards during data collection and analysis. By following an ethical code, researchers protect the interests of participants, which fosters positive engagement and contributes to the overall success of the research (Grant, 2021). Furthermore, incorporating ethics into a study allows researchers to employ specific approaches that minimize bias and create a constructive environment for participants.

Laryeafio and Ogbewe (2023) emphasize the importance of protecting participants during primary data collection. In the research focused on understanding the factors influencing the integration of educational training curricula for IL nonprofits, particularly regarding the negative impacts of social media on BBFTPTs, key ethical considerations were implemented. Ethical practices such as informed consent and confidentiality were crucial for the success of the study, as they help maintain a trusting atmosphere during the collection and use of participants' information. Grant (2021) underscores that obtaining informed consent is a key ethical consideration that aligns with both ethical and legal standards.

Ensuring participant protection and acquiring informed consent were critical steps in obtaining approval from the Institutional Review Board (IRB), thereby establishing the research's credibility and validity. To maintain confidentiality, pseudonyms were assigned to participants, labeling nonprofit leaders as L1, L2, L3, and so on. This method not only safeguarded their identities but also helped reduce bias throughout the research process. By implementing ethical considerations, researchers honor and uphold the autonomy of participants, which results in positive interactions during data collection. The study ensured that each participant provided informed consent by signing a consent form via email before the research commenced.

Chapter Summary

This chapter has developed a detailed methodological framework to explore how Illinois nonprofits consider the adverse effects of social media on their curricula of mental health training for BBFTPTs. Through the qualitative exploratory design, which was enabled by the use of purposive sampling and seven semi-structured interviews, the depth and flexibility needed to explore this underexamined phenomenon was achieved. The choice to interview nonprofit leaders instead of the vulnerable population noted in this study highlighted the ability to receive

valuable responses concerning the experience of BBFTPTs without the need for their direct participation. Semi-structured interviews and program analysis allowed for the collection of substantial amounts of data, and the use of a theme-based analysis approach by Braun and Clark (Clarke & Braun, 2017) assisted in interpreting organizational processes. Strict ethical procedures that covered informed consent, use of pseudonyms to ensure confidentiality, and IRB approval safeguarded the well-being of the participants and increased the credibility of the research. The methodological foundation of the study used a framework that produced credible, practical knowledge that can guide nonprofit practices and policymaking to manage the mental health consequences of social media on this at-risk group.

CHAPTER IV: RESULTS

This dissertation research is placed in the context of Chicago nonprofits that provide mental health services to Black and Brown female teens and preteens (BBFTPTs). It aims to increase the understanding of how nonprofit organizations develop their mental health training programs to help BBFTPTs navigate the negative impacts of social media on their mental health. To answer the research question, the study conducted semi-structured interviews with seven nonprofit leaders with expertise in serving BBFTPTs, who shared their insights into the current mental health programs and training offered by their organizations, and the curriculum development process, to ensure the effectiveness of mental health services in addressing the adverse effects of social media. This chapter presents the profiles of the interview participants and their organizations. It also explains the coding procedures used to analyze the interviews and summarizes the results based on the theoretical frameworks of institutional theory, resource dependency theory, and change management. The chapter concludes with a summary of the key findings related to each theme and a discussion of the next steps.

Data Collection

The data collection process concluded with seven participants, a sample size that aligns with Yin's (2018) recommended range of 7-10 participants for achieving data saturation in qualitative research. This number successfully reached data saturation. Furthermore, all selected participants remained actively engaged throughout the study, which enhanced its reliability. As a result, clear patterns emerged across a variety of organizations, encompassing both newer initiatives (2-3 years) and well-established programs (up to 13 years). Additionally, the participants represented a range of leadership roles, including founders, executive directors, CEOs, and program managers, providing diverse perspectives on the development of mental

health interventions that address the impacts of social media on BBFTPTs. The consistent participation rate, with zero dropouts, further validated my recruitment strategy and highlighted the relevance of the research question to these community-based organizations.

At the beginning of the interviews, all participants were briefed on the research objectives and assured that their confidentiality would be maintained through de-identification, with participants coded as Participant 1 (P1) to Participant (P7). Participants gave informed consent to engage in the Zoom interviews and respond to the questions. The duration of the interviews varied, with the longest session lasting 44 minutes and 15 seconds with Participant 1, who provided extensive organizational details. Mid-range interviews included Participant 2 at 36 minutes and 19 seconds and Participant 6 at 30 minutes and 14 seconds, offering balanced and insightful contributions. The shorter sessions with Participant 3 (29 minutes), Participant 4 (26 minutes and 16 seconds), and Participant 5 (25 minutes and 21 seconds) were concise, yet equally informative.

Analytical Approach

To ensure the effectiveness of the study, the interviews were recorded, transcribed, and subsequently coded using Microsoft Excel, in accordance with the thematic analysis framework established by Braun and Clarke (see Clarke & Braun, 2017). These methodologies were employed to identify patterns relevant to the research question concerning the impact of Illinois mental health training programs on the target population. Guided by Braun and Clarke's thematic analysis framework (Bryne, 2021), the interview transcriptions were reviewed to identify common themes using the assumptions of the three major theoretical frameworks: resource dependency theory, institutional theory, and change management theory, each illuminating distinct challenges and adaptations in nonprofit organizations serving BBFTPTs. Resource

dependency theory manifests through three interconnected themes: Funding Constraints and Capacity Limitations, Strategic Partnerships and Resource Sharing, and Grant-Influenced Programming.

These themes highlight how external resource controllers significantly shape organizational strategies, compelling nonprofits to develop adaptive approaches like partnerships and alternative funding models to overcome constraints. Institutional theory emerged through three transformative themes: cultural relevance and identity-based programming, institutional feedback and legitimacy, and adapting language and frameworks. These themes demonstrate how organizations challenge institutional isomorphism by developing culturally responsive approaches that break from traditional mental health frameworks. Change management theory was evidenced through three adaptive themes: post-pandemic mental health adaptations, youth-centered program design, and creative mental health approaches. These themes illustrate organizations' responsive evolution to address intensified mental health challenges following the COVID-19 pandemic.

Data Treatment

Microsoft Excel was utilized as the primary analytical tool to systematically sort interview transcripts into meaningful themes using a structured five-column framework. This framework included codes for the main themes related to resource dependency theory, institutional theory, change management theory, operational definitions, detailed subcodes, and participants' specific examples. This systematic approach identified nine sets of code words and phrases, firmly grounded in the participants' voices and aligned with theoretical concepts. The complete Excel coding framework used in this analysis is provided in Appendix F.

To maintain the quality of the research, several measures were put in place to ensure thorough data analysis. First, the interview questions and analyzed data were shared with the research committee for comprehensive review and recommendations. This external review offered reassurance that the research was relevant and appropriate for the chosen methodology. Second, to ensure consistency in coding the multiple interview transcripts due to the nature of qualitative data (Mattimoe et al., 2021), a coding framework was developed that provided a high level of categorization while allowing for interpretative flexibility. The coding strategies were reviewed and approved by the committee, thus enhancing inter-observer reliability (Scott et al., 2019). Third, to mitigate this risk of bias due to the subjectivity of the analysts in interpreting the results, a reflexive journal was kept throughout the analysis process, documenting my thoughts, assumptions, and biases. This practice ensured a systematic and objective approach to data analysis during the research.

Participant Description

The study employed purposive sampling to recruit seven leaders from mental health-related nonprofit organizations based in Illinois. This strategic selection process ensured involvement of professionals who have experience working with BBTPTs, understand the curriculum development process, and have deep knowledge and experience in the service delivery and management of non-profit organizations. The participants were founders, executive directors, program managers, and CEOs with diverse professional experiences in education, social work, and community development. The leadership structures vary significantly, with three participants (P4, P5, P6) explicitly identified as holding executive-level positions, demonstrating formal organizational hierarchies, while others operate with less defined leadership roles. Five participants were founders or executive directors, and three were in other

leadership roles. Table 1 presents a detailed overview of the participants and provides an organizational overview. For more details on all seven participants and organizations, see Appendix D.

Table 1

Participant and Organization Overview

Participant	Population Served	Key Services
P1	1,000+ girls per year	Holistic development, literacy, health and education
P2	Minority girls: 80 in first cohort	SEL membership, violence prevention, career prep
P3	African-American girls ages 11-17	Media literacy, storytelling, transformation
P4	Girls in 4th-8th Grade	Leadership, mentorship, entrepreneurship
P5	800 girls per year	School and city programs, life skills
P6	Over 1500+ girls	Bullying prevention, health, empowerment
P7	Women and girls	Nutrition, mindset, faith-based development

All participants were affiliated with organizations supporting vulnerable young people, particularly BBTPTs. While these organizations primarily focus on BBTPTs, several also extend their missions to include family support, literacy development, and community engagement. The organizations the participants represent have been operational for 2.5 to 13 years, with most being founded between 2014 and 2018. For instance, P3's organization has been active for 13 years and concentrates on enhancing media representations of BBTPTs through its literacy and counseling initiatives. Meanwhile, P5 and P7 oversee organizations that serve the largest

populations of girls, with each organization reaching close to 1,000 and several hundred individuals, respectively. This underscores the significant diversity within the nonprofit sector serving this demographic in Illinois.

Geographic distribution shows concentration in urban Illinois communities, particularly on Chicago's South and West sides. Organizations exhibit different operational models. P6's organization maintains in-school and weekend community programming, while P7 employs a mobile service delivery approach. Organizational capacity also varies considerably: while P5's organization serves approximately 1,000 individuals annually across multiple age groups and nine schools, others like P6 work with more limited populations (30-45 youth annually). P3's organization demonstrates interesting reach dynamics, directly serving only 35 girls but impacting approximately 600 through events. Most organizations maintain minimal staffing—P6 is the sole full-time staff member. At the same time, P4 employs one full-time and five part-time staff, reflecting resource constraints common among community-based nonprofits.

The participating organizations exhibit diverse operational scales while sharing a common commitment to serving Black and Brown girls in Chicago communities. Organization 1, established in 2018, serves approximately 2000 people annually, and takes a holistic approach to literacy support, serving families and youth on Chicago's South Side with an annual revenue of approximately $250K-$500K. With seven years of service, their comprehensive programming addresses not only literacy but also the well-being of families, recognizing that multiple factors impact children's ability to learn and develop effectively.

Organization 2, founded in 2017, empowers minority adolescent girls through comprehensive programming that bridges social-emotional learning with academic achievement. Serving an initial cohort of 80 participants with a three-member staff team, the organization

operates on donor funding from prestigious foundations, including the Obama Foundation and Chicago Community Trust. The organization uniquely combines character education with practical exposure opportunities, creating pathways for underserved Black, brown, and transgender teen girls to develop both moral leadership and professional skills for future success.

Organization 3, founded in 2012 and approaching 13 years of service, focuses specifically on African American girls, helping them critically examine social, cultural, and political ideologies in media. Their annual revenue of $514,057 allows them to serve 35 girls in person and to influence over 600 girls annually through different events. Organization 4 was founded in 2016 and officially incorporated as a nonprofit in 2021, with nearly 9 years of service empowering girls in 4th-8th grade across western and southern Illinois. The organization serves 75-100 participants annually with one full-time and five part-time staff members operating primarily through foundation grant funding and partners with organizations like Gary Comer Youth Center and Breakthrough rather than providing direct mental health services. In comparison,

Organization 5 operates on a larger scale. It was established in 2017 and now serves 800-1,000 girls each year across 8-9 schools on Chicago's South Side, with a budget of $2.7 million and 20 paid staff members. At the same time, Organization 6, established in 2014 with 11 years of service, has served more than 1,500 girls through their anti-bullying and empowerment programs for ages 8-18. Organization 7, founded in 2015 with nearly 10 years of service, operates with a mission to empower women and girls through holistic health and wellness support. With an estimated annual reach of 100 participants and a lean team of approximately three staff members, the organization employs a mobile operational model to serve underserved communities. The organization connects participants with external mental health resources

through partnerships with approximately three schools and ten community-based organizations. Organization 5 provides MHS directly with specific programming, while Organization 2 also uses partnerships to link participants to appropriate mental health resources through outsourcing arrangements.

The organizations' approaches to mental health services reflect their operational capacities, with larger entities maintaining in-house programming. In contrast, smaller organizations leverage community partnerships to ensure their participants receive necessary support. Some organizations have formalized mental health programming with trained professionals on staff, while others approach mental health through arts-based activities, mentoring relationships, or community partnerships that provide external referrals. This distinction highlights differences in capacities and philosophical approaches to youth wellness. Following the pandemic, all organizations have intensified their focus on mental health and the effects of social media, acknowledging these factors as critical influences on their target populations.

The geographical concentration of these organizations in Chicago's South and West sides highlights their commitment to serving historically underserved communities. While most maintain physical locations in specific neighborhoods, organization7 utilizes mobile service models to expand their reach. This collective presence represents a significant community-based infrastructure supporting Black and Brown girls' development in areas where mainstream services have traditionally been limited or culturally misaligned with community needs. To obtain a more comprehensive understanding of these organizations, see the table in Appendix E.

Results

In conducting interviews with seven nonprofit leaders, qualitative analysis revealed six primary themes and three sub-themes explaining how organizations grapple with the complex challenges and employ innovative approaches to address the mental health needs of BBFTPTs regarding social media. As shown in Table 2, these themes exhibit varying degrees of participant recognition and theoretical alignment concerning three key frameworks: resource dependency theory, institutional theory, and change management theory.

Table 2

Emergent Themes, Theoretical Alignment, Participant Acknowledgement, and Number of Occurrences

Main Themes	Theoretical Alignment	Shared by	Occurrence (out of 7)
Funding Constraints and Capacity Limitations	RDT*	P^ 1, 3, 4, 5, 6, & 7	6
Cultural Relevance and Identity-Based Programming	IT**	All	7
Institutional Feedback and Legitimacy	IT	All	7
Post-Pandemic Mental Health Adaptations	CMT***	All	7
Youth-Centered Program Des	CMT	All	7
Creative Mental Health Approaches	CMT	All	7
Sub-Themes	**Theoretical Alignment**	**Shared**	**Occurrence (out of 7)**
Adapting Language and Frameworks	IT	P 1, 2, 3, 4, & 5	5
Strategic Partnerships and Resource Sharing	RDT; IT	P 2, 3, 5, 7	4
Grant-Influenced Programming	RDT	Ps 1, 3, 5, & 6	4

* Resource dependency theory (RDT
** Institutional theory
*** Change management theory
^ Participant

Theme 1: Funding Constraints and Capacity Limitations

Resource dependency theory illuminates the critical challenges faced by nonprofit organizations addressing mental health for BBFTPTs. Across the interviews, more than half of the participants explicitly highlighted funding as a paramount constraint that directly impacts their ability to develop comprehensive mental health programs. This observation highlights the underlying resource dependency that these organizations face, where external funding sources determine their operational capabilities. The interviews revealed that funders often impose restrictive conditions that limit program flexibility, with Participant 5 noting that "unfortunately, the funding determines the neighborhoods for some of the funding, so it determines the neighborhoods, and then it also could determine like what we are focusing on." Such constraints demonstrate how external resource controllers can significantly shape organizational strategies and service delivery.

The capacity constraints due to a lack of funds are reflected in various aspects of program development and implementation. A few participants explicitly mentioned the need for more staff and professional expertise to address mental health issues effectively. Participant 4 candidly expressed this limitation, stating, "I would like to have a consistent professional on the team to help build out the mental health curriculum. But we're not there just yet." These insights show how the scarcity of resources directly affects the ability of an organization to offer comprehensive support. The organizations are compelled to make strategic compromises, including collaborating with other institutions, creating alternative funding strategies, or reducing the scope of their interventions. Participant 1 illustrated this adaptive approach, sharing how they initially launched a pilot workshop without funding, hoping to demonstrate value and secure future resources: "For the beginning workshop, because it was a pilot. She didn't ask for funding. She actually just gave her time. […]. And now let's take this model to get more funding."

Table 3

Theme 1 Participant Testimonies

Participant ID	Testimony
P5	"The desire to go deeper with more funding and actually offering free therapy to our students... But we haven't had the funding to be able to accommodate that New Year."
P1	“Funders don't always understand that. Like, you may say I want you to, even if it's like a math program. But even there should be some level of like Sel work being done.”
P4	“Because funding has always been so hit or miss, we've never been able to truly rely on it. We. We just set out to do our work. And then, hopefully, you know, we get the funding that will support it.”

The resource dependency becomes most apparent in the organizations' ability to address complex mental health challenges, particularly those related to social media's impact on BBFTPTs. Almost all participants highlighted how funding limitations restrict their capacity to develop comprehensive, innovative interventions. Participant 3 articulated specific growth needs, including "more research that is generated from the perspective of our participants," "policy implementation," and curriculum development, which require substantial financial investment. The organizations are essentially caught in a cyclical challenge where limited resources prevent them from fully addressing the mental health needs they've identified. Participant 5 emphasized this point, noting that "the need is now greater, the resources are scarcer." This resource dependency creates a significant barrier to providing holistic, culturally responsive mental health support. Some organizations have developed creative workarounds, such as utilizing volunteer expertise, creating peer support networks, and leveraging community partnerships. As presented in Table 3, testimonies from participants indicate how funding constraints pervasively affect organizational capacity and program development. However, these strategies, while innovative, cannot fully compensate for the fundamental resource constraints that limit their ability to

provide comprehensive mental health interventions for Black and Brown female teens and pre-teens.

Theme 2: Cultural Relevance and Identity-Based Programming

All seven participants emphasized the critical importance of culturally relevant programming that affirms the identities of Black and Brown female youth. This approach directly challenges institutional isomorphism by breaking away from traditional mental health frameworks that fail to address the unique experiences of marginalized communities. Participant 3 established this foundation clearly: "Our primary factor is... from a Black woman's historical perspective, and we use that as the baseline […] we want to make sure that it's culturally relevant." This sentiment was reinforced by Participant 2, who noted the delicate balance between protection and identity affirmation: "The biggest pushback that we get from students when we do the social media work is helping them to understand that we're not... interested in policing their identities but we are interested in protecting their identities." Participants consistently reported that culturally specific approaches were essential for dismantling outdated institutional structures that have historically mischaracterized Black and Brown girls' mental health needs.

A majority of the participants described how identity-based programming directly counters harmful social media narratives by empowering youth to reclaim their narratives. Participant 3 stated: "We work with girls around media literacy and storytelling, helping them to dispel those stereotypes and then to reclaim their own narratives." Similarly, Participant 1 highlighted the fundamental identity struggles: "I always think about... especially when it comes to being a Black girl. It's already challenges of just like if who I am is considered pretty or pretty, or good enough." By reshaping these institutional frameworks, organizations create spaces that

validate experiences otherwise marginalized by mainstream approaches. Participant 6 articulated how mental health needs have been historically misinterpreted: "The communication that comes from a lot of Black and Brown girls... does not look like the cookie-cutter, nicely packaged communication... and so people have labeled it as y'all have attitudes." These findings demonstrate how institutional theory's emphasis on dismantling outdated structures is operationalized through culturally responsive programming.

Table 4

Theme 2 Participant Testimonies

Participant ID	Testimony
P3	"Works specifically with Black girls and those who are impacted by the negative stereotypes that are portrayed in media, the sociological, ideological, and political messages."
P2	"We teach girls to identify the negative messages, and then we help them create positive messages. So, whether they're doing storytelling or they're doing spoken word, or they're doing a podcast, they recognize what is negative or what's stereotypical…"
P6	"In terms of implementation, we do provide facilitator training. So, we ensure that our facilitators are aware of the level at which we need to give this information... Cultural relevancy for me is like the top of the tops."

Representative participant testimonies, in Table 4, display how culturally relevant, identity-affirming approaches are central to their programming frameworks. All but one of the participants described implementing specific identity-affirming practices that serve as institutional innovations, breaking from traditional mental health approaches. Peer support models featured prominently, with Participant 6 noting: "We have really increased our mentor groups... with one mentor, and then we have some guiding questions that just kind of help them become honest and vulnerable and transparent." Participant 4 highlighted community-based

celebrations: "We've adjusted across all our sites to do a mental health check-in before we start our session." These approaches represent what institutional theory would identify as critical adaptations necessary to overcome internal barriers to effective service delivery. Participant 5 emphasized youth voice as central to this institutional transformation, "Making sure that youth voice is at the center of it is definitely a best practice." By centering programming in cultural relevance and identity affirmation, these organizations challenge institutional isomorphism and develop innovative approaches that more effectively address the unique mental health needs of Black and Brown female youth navigating social media's challenges.

Theme 3: Institutional Feedback and Legitimacy

All seven participants identified institutional constraints that shape how their organizations respond to the mental health needs of BBFTPTs in relation to social media. These constraints manifest through funding limitations, organizational structures, and societal expectations. Participant 5 explicitly noted how resources determine program focus: "Unfortunately, the funding determines the neighborhoods for some of the funding, […], and then it also could determine like what we're focusing on." This institutional pressure affects program legitimacy, as organizations must balance funder expectations with community needs. Participant 1 reinforced this challenge: "Funders don't always understand that... even if it's like a math program. But even there should be some level of like SEL [social emotional learning] work being done." These quotes illustrate how institutional factors create feedback loops that can either enable or constrain organizations' abilities to effectively address the unique mental health challenges facing their target population.

The interviews revealed that organizations are actively working to establish legitimacy through responsive curriculum development processes that challenge traditional institutional

approaches. Almost all participants described using feedback mechanisms to ensure their programs remained relevant and effective. Participant 7 emphasized evidence-based approaches: "We have professionals that we, of course, consult with. We do research. So, a lot of stuff is applied evidence based." Similarly, Participant 1 highlighted the importance of community responsiveness: "I'm very big about making sure that our programming is in real time responsive to the families and kids we're serving." These organizations are creating feedback loops with their communities that validate their approaches while potentially disrupting institutional isomorphism. Participant 3 explicitly called for "more research that is generated from the perspective of our participants and us as community members versus outside research," challenging the traditional knowledge hierarchies that can limit program innovation. Participant testimonies provided in Table 5 pinpoint how organizations construct legitimacy through responsive programming and collaborative partnerships in managing institutional constraints.

Table 5

Theme 3 Participant Testimonies

Participant ID	Testimony
P1	“We've also implemented the Ambassador program, and that is where we have girls who are in the organization, who act as like representatives who act kind of as the liaisons, the voice of our girls with our staff”
P2	“I'm very big about making sure that our programming is in real time responsive to the families and kids we're serving. So it influences. Maybe the books we pick, or even the topics that we're discussing in the different workshops.”
P7	“And in our partnerships with our schools the we do a I guess, an intake form from the actual school, and then they'll tell us what the issues are that they see also in their school community.”

Institutional legitimacy is further achieved through collaborative partnerships that help these organizations overcome resource limitations while maintaining cultural relevance.

Participant 2 described a partner-based approach: "For girls who have serious concerns, girls and families that have serious concerns, we partner them with organizations that have a focus on clinical support therapy things like that." This networked approach allows organizations to maintain legitimacy while acknowledging their limitations. Participant 7 emphasized cross-organizational collaboration: "The partnerships, the coming together with other organizations with other like minds, I think we will have a greater impact as a whole." These partnerships help overcome what Participant 6 identified as the primary resource constraint, stating that, "Money. I hate to say it... but it's money, it's money, and it's manpower. But money begets manpower." By developing these institutional networks, organizations can challenge traditional program structures while maintaining legitimacy with funders, stakeholders, and most importantly, the BBFTPTs they serve.

Theme 4: Post-Pandemic Mental Health Adaptations

Social media's pervasive influence on mental health during the COVID-19 pandemic emerged as a significant concern during the interviews. All participants stated that their organizations documented a significant shift in mental health challenges faced by BBFTPTs following the COVID-19 pandemic, highlighting that “all of the needs that were already existing were exacerbated” (participant 5). The respondents noted that the pandemic conditions amplified pre-existing vulnerabilities, including anxiety, depression, and body image issues. Participant 3 clearly articulated this transformation: “Before the pandemic, you had a lot of anger from Black girls […]. After COVID, what we saw was […] a lot of self-harm, a lot of depression […].”

All participants attributed the spike in mental health needs to the increased social media exposure during the isolation period. As Participant 5 emphasized that “social media has also amplified things as well. So what girls are seeing and experiencing online, especially since so

much of their socialization was moved to online, you know, in the aftermath of the Covid pandemic." This observation was echoed by Participant 3, who noted that "social media is, […] unfortunately, the first gateway to mental health consumption." Participant 1 also highlighted the link between increased social media culture and mental health issues, stating that, "Because there are so many […] additional platforms that they [girls] can look at […]. They will be sad about things."

In response to these evolving challenges, organizations implemented innovative adaptations to their mental health support frameworks. A majority of the participants described creating new program components specifically addressing post-pandemic needs. Participant 4 stated: "We've adjusted across all our sites to do a mental health check-in before we start our session...The first ten minutes are dedicated to just checking in on the girls mentally." Participant 5 took a structural approach, reporting: "We created a mental health counselor position. So, we actually created a position with a counselor who does groups for girls across our school sites and in the community." These organizational shifts demonstrate the application of change management theory through responsive program development. Participant 7 detailed additional therapeutic elements: "We've incorporated meditation [and] different activities like coloring," while Participant 3 implemented "group therapy sessions and healing circles with mothers and daughters," addressing intergenerational trauma as part of their holistic approach to post-pandemic recovery.

The integration of social media literacy has emerged as a necessary adaptation across all interviewed organizations. Almost all participants explicitly identified social media as a new theme included in their programming after the pandemic. Some organizations responded by implementing media literacy training in which they were teaching "[…] why messages are

created, who creates the message, and what the intended outcomes should be" (Participant 3). Participant 1 shared that they included in their curriculum topics that would teach youth how to use social media as a tool and how they could control social media to prevent it from having a negative influence on their mental health. Participant 6 stated that they included social media awareness in their curriculum whenever the opportunity arose.

In Table 6, key participant testimonies primarily emphasize noteworthy variations in the appearance of mental health manifestations and the creation of innovative organizational responses to mental health in the post-pandemic landscape. These examples reflect a deliberate change management strategy that recognizes the influence of social media on mental health. Additionally, they aim to equip young people with critical thinking skills, alternative activities, and community ties to help lessen the adverse effects of social media in the post-pandemic world.

Table 6

Theme 4 Participant Testimonies

Participant ID	Testimony
P3	"Before the pandemic, you had a lot of anger from Black girls... After COVID, what we saw was a lot of internalized processing of emotions. And so, a lot of self-harm, a lot of depression, a lot of reclusiveness, disconnecting is what we saw after COVID."
P4	"Before the pandemic. I think it was more so about trauma growing up in a trauma-induced environment... after COVID, the priority for mental health. I think it became very much so about isolation."
P5	"We created a mental health counselor position. So, we actually created a position with a counselor who does groups for girls across our school sites and in the community."

Theme 5: Youth-Centered Program Design

Youth-centered program design emerged as a pivotal theme across all interviews, with all participants emphasizing the importance of responsive, participant-driven approaches in addressing the mental health challenges BBFTPTs face regarding social media. The post-pandemic landscape has necessitated significant programmatic shifts, as Participant 1 noted, "I'm very big about making sure that our programming is in real-time responsive to the families and kids we're serving." This responsiveness involves systematic feedback mechanisms, with Participant 1 describing how they gather input "through formal and informal surveys" and "just in conversation." Similarly, Participant 5 emphasized: "Making sure that youth voice is at the center of it is definitely the best practice." This theme highlights how effective change management requires programs to evolve based on direct input from the youth they serve rather than imposing predetermined structures.

The interviews revealed specific youth-centered strategies that organizations have implemented to address evolving needs. Participant 4 described adapting their approach to include "mental health check-ins at the beginning of sessions... The first 10 minutes is dedicated to just checking in on the girls mentally." Several participants highlighted peer leadership models, with Participant 6 explaining: "We've also implemented the Ambassador program... where we have girls who are in the organization […] who act kind of as the liaisons, […]." These approaches reflect a fundamental shift from traditional top-down programming. Participant 3 emphasized the importance of youth-led content creation, stating: "We use it for podcasting again, reshaping the narrative that they can tell stories of their own that are positive." This approach empowers youth not just as recipients of services but as active participants in designing solutions.

Program diversity emerged as another critical element of youth-centered design, with organizations developing varied approaches to engage different learning styles and interests. Participant 6 advocated for "diversity in programming... So, we have heavy dialogue sessions, and then we also have heavy activity sessions. […]. We have community service projects." This multi-faceted approach helps reinforce learning through varied experiences. Participant 7 described how they customize activities based on youth preferences: "What gifts do you believe you have? Like some like to draw, some like to sing, some like to create music..." Several participants emphasized the importance of creating safe spaces for authentic expression. Participant 1 observed powerful peer connections when implementing "healing circles," where girls experience "a level of sisterhood and feeling as if there are others that can connect to me."

These findings suggest that effective change management in youth mental health programming must center youth voices, foster peer leadership, and offer diverse engagement strategies that respond to participants' unique needs, interests, and cultural contexts. Table 7 showcases stories that demonstrate how organizations elevate the voice of youth and ensure responsive, participant-led programming approaches.

Table 7

Theme 5 Participant Testimonies

Participant ID	Testimony
P4	"Mental health check-ins at the beginning of sessions... The first 10 minutes are dedicated to just checking in on the girls mentally."
P6	"Best practices are a variety of diversity in programming... So, we have heavy dialogue sessions, and then we also like, have heavy activity sessions. We have field trips. We have community service projects."
P3	“Making the connection with the girls is not always easy. And the age gap between us and the girls... So, let's find a way to bridge this gap between us so that we can get a better understanding of why social media plays such a factor for them.”

Theme 6: Creative Mental Health Approaches

The interviews revealed that all participants implemented creative and innovative approaches to mental health support for BBFTPTs, particularly in response to the pandemic's impact and social media challenges. These organizations recognized that traditional mental health approaches were insufficient and developed holistic interventions. Participant 3 emphasized media literacy as a critical mental health intervention, stating, "We understood and understand that social media is really unfortunate, the first gateway to mental health consumption." Similarly, Participant 1 highlighted a comprehensive approach: "Our focus is to really provide families with support when it comes to literacy. […]... not just worried about their ability to read and write […]." These creative approaches acknowledged the complex interplay between social media exposure, cultural identity, and mental well-being, moving beyond conventional counseling to address the root causes of distress.

The interviews highlight that a majority of the participants incorporated expressive arts and creative outlets as crucial components of their mental health programming. Participant 4 described implementing "mental health check-ins" and "rose and thorn" exercises to identify positives and challenges, while Participant 7 stated, "We've incorporated meditation...We've incorporated different activities like coloring." Participant 1 emphasized the value of creative writing: "We started doing more like writing workshops... That's also sent around like journaling, and so, being able to give them tools they can use when they leave us." Participant 3 uniquely leveraged creative media production as a therapeutic intervention: "We use it for podcasting again, reshaping the narrative that they can tell stories of their own that are positive." These approaches represent a significant shift from traditional mental health services, recognizing that creative expression provides accessible entry points for addressing complex emotional challenges while building essential coping skills and strengthening identity formation.

All participants emphasized community-building and peer support as innovative mental health approaches. Participant 6 noted, "We definitely wanted to amplify the idea of community building a sense of community, because that was something that we lost in the pandemic.". Participant 6's organization has implemented group mentorship: "We have them based on age in larger groups with one mentor, and then we have some guiding questions […]." Participant 3 described intergenerational healing approaches: "We began to do group therapy sessions and healing circles with mothers and daughters." Participant 1 highlighted the power of shared experiences: "To hear somebody that you probably were comparing yourself to say that they feel insecure...you're like, what really do you do?" Participant 5 emphasized the youth voice: "Making sure that youth voice is at the center of it is definitely a best practice." These approaches recognize that healing occurs within community contexts, utilizing collective wisdom and shared experiences to normalize discussions around mental health and reduce stigma.

The organizations faced funding challenges while implementing these creative approaches, with more than half of the participants explicitly mentioning resource limitations. Participant 1 explained, "For the beginning workshop, because it was a pilot. She didn't ask for funding. […]. But the hopes were we [would be] able to see does this work." Participant 6 was direct: "Money. I hate to say it... but it's money, it's money, and it's manpower. But money begets manpower." Despite these constraints, the organizations demonstrated remarkable adaptability and innovation. Participant 2 emphasized relationship-building as foundational: "Organizations have to be very intentional and strategic about trust building between facilitators and... the constituency." Participant 5 created new positions: "We created a mental health counselor position...who does groups for girls across our school sites."

Table 8 shows participant testimonies of the innovative and holistic mental health interventions implemented by these organizations. These creative approaches represent a paradigm shift in addressing the mental health needs of Black and Brown female youth, moving beyond deficit-based models toward strengths-based, culturally affirming interventions that empower participants to navigate social media challenges and develop positive identities.

Table 8

Theme 6 Participant Testimonies

Participant ID	Testimony
P1	"We've used reading as, yes, a way to do better in school, communicate all those things, but an alternative to maybe lashing out or feeling like you have to like close up right."
P3	"We ask them also to create their own social media, using positive messages. We use it for podcasting again, reshaping the narrative that they can tell stories of their own that are positive."
P6	"So, sitting down the coloring techniques, the meditation, the steps of learning how to let go so that's dealing with past trauma and paying that a woman or girl, of course"

Sub-Theme 1: Adapting Language and Frameworks

The theme of Adapting Language and Frameworks emerged prominently across the interviews, revealing how organizations are dismantling traditional mental health approaches to better serve BBFTPTs in navigating social media challenges. Almost all participants emphasized the necessity of adapting institutional language to create more culturally responsive frameworks. Participant 3 highlighted this by noting, "Our primary factor is this from a woman's, a black woman's, […], and we use that as the baseline […] teaching our girls, we want to make sure that it's culturally relevant." Similarly, Participant 2 articulated the tension between protecting identities while honoring developing self-concepts: "The biggest pushback that we get from

students when we do the social media work is helping them to understand that we're not...interested in policing their identities, but we are interested in protecting their identities." This adaptation reflected what institutional theory identifies as breaking away from isomorphic pressures to create more effective interventions.

The interviews revealed how organizations are reimagining traditional mental health frameworks through innovative language around media literacy and creative expression. Participant 3 emphasized social media as "the first gateway to mental health consumption," necessitating new frameworks like "media diaries" and "Media Bingo" to help girls critically analyze content. Participant 1 described shifting from deficit-based language to strength-based approaches: "Social media is a tool, and it's a tool that we control... […], and we just are received like we have nothing that we can control about it." More than half of the participants explicitly described reframing social media discourse from one of fear to one of empowerment. Participant 5 noted the critical importance of "media literacy, allowing them to see what is to discern what's real, what's not real." These adaptations represented institutional theory's concept of dismantling outdated structures that no longer served the population's evolving needs.

The data further illustrates how organizations are challenging institutional barriers by advocating for funding models that support holistic, culturally responsive frameworks. Almost all participants identified funding constraints as limiting their ability to fully adapt programming languages and frameworks. Participant 1 stated, "Funders don't always understand that... […]. But even there should be some level of like SEL [social emotional learning] work being done." Participant 4 similarly expressed the need for "connecting with organizations, with funders and founders of social media marketing agencies and social media tools."

These perspectives aligned with institutional theory's emphasis on overcoming external barriers to change. Participant 3 further articulated the need for systemic change: "I also think there's a real need for us to begin to implement policy... so that it's not just about their change behavior individually, but about how they can also influence policy." These findings demonstrated how organizations were working to reshape institutional frameworks at both programmatic and systemic levels.

Testimonies presented in Table 9 show how organizations are rethinking the old mental health language and frameworks to improve their target population.

Table 9

Sub-Theme 1 Participant Testimonies

Participant ID	Testimony
P3	"Our primary factor is this from a woman's, a Black woman's, historical perspective, and we use that as the baseline for anything that we're teaching our girls."
P2	"When we're going into the CTS schools, unfortunately, a lot of times we get pushed back. We get it's like a negative experience, so we don't allow them to really implement much."
P3	"So, one of the things I do, and even with like parents, when we do a workshop, is just understanding that social media is a tool, and it's a tool that we control."

Sub-Theme 2: Strategic Partnerships and Resource Sharing

Almost all participants explicitly highlighted the critical nature of collaborative relationships to address resource limitations. As Participant 6 candidly stated, "We want to do like we have so many like college tours... We would love to do extended trips like things of this nature, and we would love to hire just more people to be in space with the girls, more facilitators." This observation underscores the external constraints organizations face,

demonstrating how resource dependencies directly shape programmatic capabilities. The interviews reveal a consistent pattern of organizations seeking external partnerships to compensate for internal resource constraints, particularly in addressing the complex mental health challenges exacerbated by social media's impact on young women of color.

The strategic partnerships manifest through diverse collaborative approaches, with organizations deliberately creating networks to enhance their service delivery. Participant 2 emphasized their approach of partnering with specialized organizations: "For girls who have serious concerns, […], we partner them with organizations that have a focus on clinical support therapy, things like that." Similarly, Participant 7 highlighted the importance of inter-organizational collaboration, noting,

> One thing I would definitely say is the partnerships, the coming together with other organizations with other like minds, I think we will have a greater impact as a whole if we reached out and touched another organization and crossed the state lines […].

These partnerships serve multiple critical functions: accessing specialized expertise, extending service reach, and mitigating resource limitations. The organizations realize that one institution cannot entirely resolve the mental health challenges of BBFTPTs, so a collaborative network of support is needed.

The resource dependency theory outlines important factors that influence the survival and effectiveness of mental health organizations. Most participants openly talked about the difficulties with funding and the need to share resources strategically. Participant 3 articulated this need directly: "We definitely always need funding because that's all of the things I just named for. You require someone to do that work and to head that up." The interviews consistently demonstrate that organizations must strategically navigate external resource

dependencies by developing innovative partnerships, seeking alternative funding sources, and creating flexible programming models. Participant 7 emphasized the critical nature of resources, stating bluntly, "Money. I hate to say it... But it's money, and it's manpower."

These strategic partnerships extend beyond financial resources to include knowledge exchange, professional development, and collective problem-solving. By working together, these organizations can reduce the impact of limited resources, which helps them better support BBFTPTs in dealing with complex mental health issues related to social media. As shown in Table 10 below, participant testimonies also demonstrate the importance of collaborative relationships in leveraging resource weaknesses and assisting in service delivery.

Table 10

Sub-Theme 2 Participant Testimonies

Participant ID	Testimony
P7	"Now, within the last year, now learning how to apply for funding and things like that, we're beginning to incorporate it, but I'm very grateful that we already have in place a system."
P6	“Our school partners are. They're like paramount when it comes to forming the curriculum, because we definitely want to make sure that we respond to the needs.”
P3	“We partner with mental health professionals; we have a partnership so that our girls can receive one-on-one therapy or group therapy.”

The emergence of strategic partnerships among mental health organizations serving BBFTPTs as a survival mechanism and effectiveness strategy embodies core principles of institutional theory in direct practice. The partnerships manifest through deliberate networking strategies, with organizations like Participant 2 partnering "with organizations that have a focus on clinical support therapy" to address specialized needs beyond their capabilities. Through institutional theory, these collaborations offer insight into how organizations are responding to

these environmental constraints by communicating effectiveness through resource optimization and service expansion, as a means to gain legitimacy and survive.

Sub-Theme 3: Grant-Influenced Programming

The interview results showed that grant-influenced programming is complex, with participants frequently mentioning the restrictions that funding structures impose. Almost all the participants in the interviews specifically mentioned that funding is a key factor in determining how organizations function and what programs they develop. Participant 5 candidly stated, "Unfortunately, the funding determines the neighborhoods for some of the funding, […], and then it also could determine like what we're focusing on." This observation is consistent with resource dependency theory, which states that external resource controllers play a major role in determining organizational strategies and priorities. The reliance on grants results in a complex situation in which nonprofits must manage their mission-driven priorities together with the particular demands and restrictions from funders.

Participant 6 directly articulated the resource constraints, stating, "funding because I know that you should... you need the financing, because in order to have a mental health therapist, […]. They need to be paid." This dependency presents a major challenge because organizations must prove their programs work while also adhering to the strict rules of grants, sometimes limiting the scope of mental health interventions for BBFTPTs.

The influence of grant-funded programs reaches beyond financial boundaries, significantly altering how organizations provide mental health services. Participant 5 highlighted this dynamic, noting that "funders often dictate program priorities, thereby limiting nonprofits' flexibility to focus on social media-specific health issues." This observation is in perfect agreement with resource dependency theory's perspective on how external funders shape

organizational decision-making. Participant 3 further emphasized the need for more community-driven research and funding, stating, "One thing is more research that is generated from the perspective of our participants and us as community members versus outside research."

The participants consistently demonstrated a tension between their deep understanding of community needs and the external constraints imposed by funding sources. The testimonies in Table 11 reveal the various dynamics between funding needs and mission-driven programming priorities. As one participant succinctly summarized, the challenge lies in maintaining program integrity while meeting the specific requirements of grant providers, highlighting the delicate balance nonprofits must strike in addressing mental health challenges for BBFTPTs.

Table 11

Sub-Theme 3 Participant Testimonies

Participant ID	Testimony
P5	“And then I say, staff, but that goes back to the funding, right? If we have the funding, then we could have the staff.”
P1	“The outside factors are things that maybe directly impact my work. Because, for example, we've had to be creative this year of funding, just because a lot of priorities of people have shifted right”
P6	“We want to do like we have so many college tours. They are great ways to engage our girls and again give them windows, and we do need the resources to ensure.”

Additional Insights

Nonprofit organizations working with young women have noted specific factors that contribute to these young women’s mental health challenges, and social media has been identified as a factor that worsens the situation. This culture involves the prejudice and marginalization of mental health in the Black community, which in most cases leads to the labelling of emotional actions as troublesome behavior, but is actually a reflection of a troubling

emotion. These challenges have been compounded by social media platforms, which have become spaces that allow discrimination based on race, body shaming, and discrimination based on stereotype (Choukas-Bradley et al., 2022; Park et al., 2023). The COVID-19 pandemic exacerbated these interactions by shifting much of young women's social engagement to virtual platforms and exposing them to more harm (Marciano et al., 2022). Participants' narratives reiterated how these social platforms affect BBFTPTs, especially as they flagged various intersectional concerns that mental health interventions have in the past excluded. Reducing these young women's behaviors to being "spicy" or having "attitudes" erases the possibility of understanding them and addressing the psychological issues they face. These digital landscapes not only impose restrictive external expectations but also internal struggle over the self and value.

Noting the limitations of the traditional approaches of training in the mental health field, these organizations have incorporated diversity and complex methods that have surpassed the simple medical model. Media literacy becomes a powerful tool and practice through which young women watch and create media content and shift from being passive users to active ones who can argue against unhealthy messages. Targeted interventions have been put in place to ensure that expression is safely facilitated, mainly through the use of healing circles, storytelling, as well as peer support groups. These interventions are purposely planned for the BBFTPTs to grapple with mental health concerns while acknowledging the intersection between social media use, race, and mental health. Through empowerment, cultural relevance, and the overall personal and social growth of an individual, these nonprofit organizations prepare the young women to handle the rising demands of the digital environment. The approaches also acknowledge that the

idea of seeking mental health support is different and should not be generalized for BBFTPTs due to social and cultural differences and their circumstances.

Findings

This study aims to understand how Chicago nonprofits that offer mental health training programs to BBTPTs incorporate the negative effects of social media into their curriculum to help their beneficiaries through the theoretical assumptions of the institutional, resource-dependency, and change-management theories. Resource dependency theory reflected how limited finances directly influence organizations' ability to design extensive mental health programs, while external funders pressure programs with a narrow scope. Organizations overcome these restraints through strategic partnerships, resource sharing, and grant-based programming. Institutional theory brought to light how organizations integrate culturally suitable, identity-based programming that contradicts the traditional mental health paradigm, build legitimacy with flexible curriculum design, and alter language to develop more effective interventions for BBFTPTs. The change management theory uncovered significant post-pandemic mental health adaptations, with organizations observing a change from externalized to internalized emotional processing in the youth. Organizations responded by adopting youth-centered program design strategies, mental health check-ins, peer leadership models, and various engagement methods. Organizations, however, also evolved innovative mental health approaches such as expression arts, community development, as well as peer support pathways that go beyond typical counseling to explore the complex interplay between social media exposure, cultural identity, and mental well-being.

The interviews highlighted that media literacy could be said to have become an important type of intervention because organizations started to create programs that turn young women into

creators, not just consumers of information, who can identify toxic content. Participants pointed out the need to educate young people on how to counter the effects of media through critical thinking, thus coping with problems such as body dysmorphia, cyberbullying, and racism. Ambassador initiatives, storytelling workshops, and podcasts are being used by organizations that focus on helping to influence a new positive identity for girls and contribute to their emotional well-being. These interventions are purposefully designed to present culturally appropriate support that emerged from the existing frameworks of traditional mental health models that are often culturally insensitive to the experiences and needs of BBFTPTs in complex digital spaces.

Chapter Summary

This chapter presents the results of interviews collected from seven nonprofit managers and leaders to increase the understanding of how Chicago nonprofit organizations develop their mental health training programs to help BBFTPTs navigate the negative impacts of social media on their mental health. From the thematic analysis, three major theoretical frameworks for non-profit organizations serving BBFTPTs were identified. The analysis revealed several patterns, including inconsistencies in the implementation of mental health training across corporate social media platforms, the importance of cultural competence, and the emergence of new interventions. The next chapter will discuss the research results, including the practical implications of the findings and recommendations for policymakers, nonprofit managers, and scholars.

CHAPTER V: CONCLUSIONS AND ANALYSIS

This dissertation research was guided by the central question: How do Illinois nonprofits that offer mental health services for youth, particularly for Black and Brown female teens and preteens (BBFTPTs), integrate the negative influence of social media topics in their educational training curricula? Insights from seven Illinois-based nonprofit organizations are interpreted through the theoretical frameworks of resource dependency theory, institutional theory, and change management theory. The findings suggest that curricular decisions are shaped by environmental pressures, organizational capacity, and adaptive learning processes. This chapter discusses the results from the cross-case analysis, providing a synthesis of how each theme manifested across the seven cases, while situating these findings within broader debates in nonprofit management, curricular innovation, and culturally responsive education. Furthermore, the chapter offers theoretical and practical insights for nonprofit practitioners, policymakers, and researchers, which are followed by study limitations and future research directions.

Overview of Study

Mental health among BBFTPTs has deteriorated notably following the COVID-19 pandemic. Research in both academic and medical fields has linked this concerning trend to the increased use of social media. The prevalence of social media usage among adolescents has created unique mental health challenges for this demographic, including cyberbullying, body shaming, and racial discrimination, which can lead to anxiety, depression, and suicidal ideation (Park et al., 2023). Despite the Centers for Disease Control and Prevention (CDC) reporting that 60% of female students experience persistent sadness or hopelessness, current nonprofit mental health training programs in Illinois inadequately address these specific issues. As Volpe et al. (2021) highlighted, racist ideologies and systemic biases stemming from online platforms place

vulnerable BBFTPTs at risk. The stigmatization of mental health in Black communities, combined with identity and validation struggles experienced by young BBFTPTs, creates a unique intersection of vulnerabilities that this research aimed to address. In response to the vulnerability of BBFTPTs, several policy initiatives and support services have emerged to address the impact of social media on youth mental health. While government and nonprofit organizations offer a variety of mental health services, it is important to note that not all of these programs specifically cater to the needs of BBFTPTs in mitigating the adverse effects associated with social media usage.

This study focused on the curriculum development process and the contextual factors that influence organizational decisions regarding the inclusion of social media-related mental health content. Semi-structured interviews were conducted with leaders from three nonprofit organizations operating in urban communities across Illinois, seeking to understand the decision-making processes behind curriculum development, particularly in relation to the inclusion of social media-related mental health content. Thematic analysis was applied to identify patterns across participant responses, using resource dependency theory (Pfeffer & Salancik, 1978), institutional theory (Meyer & Rowan, 1977), and change management theory (Kotter, 1996) as analytical lenses. The resource dependency theory helped explain how funding sources and donor expectations impact programming decisions, while institutional theory provided insight into how legitimacy and alignment with established norms might influence organizational behavior. In contrast, change management theory helped us understand how nonprofits adapt to transformations in response to crises, such as the pandemic. Together, these theoretical frameworks allowed for a deeper understanding of how external stakeholder pressures, funding

constraints, institutional norms, and internal adaptive capacity influence curricular decisions in nonprofit mental health programs.

The analysis revealed both commonalities and distinctions across the cases, shedding light on how organizations navigate the intersection of social media's impact on mental health, youth development, and community-based service delivery. The findings contribute to ongoing conversations about trauma-informed care, culturally responsive education, and the role of nonprofits in addressing emerging mental health challenges in the digital age.

Analysis

The findings of this study revealed that Illinois-based nonprofits serving Black and Brown girls have adopted a range of strategies to address the negative mental health effects of social media within their training curricula. Across the seven participating organizations, there was a shared recognition that the mental health challenges facing girls have intensified in the aftermath of the COVID-19 pandemic, with social media functioning as both a contributing factor to psychological distress and a space for youth identity development. The organizations integrated social media topics through trauma-informed, culturally responsive, and youth-centered programming, often using creative modalities such as storytelling, podcasting, and visual media to help participants critically analyze and reframe online content.

Curriculum development was driven by community input, professional expertise, and ongoing feedback mechanisms, though each organization faced constraints related to staffing, funding, and technological access. Strategic partnerships with schools, mental health providers, and peer organizations played a crucial role in expanding capacity and supporting program implementation. While most organizations sought to maintain autonomy in curricular decisions, funder influence remained a significant factor in shaping geographic reach and program

emphasis. Despite these challenges, all organizations demonstrated a commitment to adaptive learning, continuous evaluation, and alignment with institutional expectations and youth needs.

Responding to the Social Media Landscape

The results provide insight into a crucial shift in mental health presentations among BBFTPTs in the aftermath of the COVID-19 pandemic, where organizations were reporting a foundational shift in externalized anger to internalized emotional processing manifesting in self-harm, depression, and social withdrawal. This change is consistent with the available literature that shows the correlation between more screen time and worse psychological health in adolescents (Twenge & Campbell, 2018) but goes beyond association to show how the isolation caused by the pandemic exaggerated pre-existing vulnerabilities by increasing social media exposure. The participants' unanimous observation that all of the needs that already existed were worsened indicates that the pandemic did not create mental health issues but acted as a catalyst, and the key driver of this catalysis was social media.

Organizations have reacted with institutional changes, such as establishing dedicated mental health counselor roles, introducing required mental health check-ins, and incorporating therapeutic practices like meditation and healing circles. These organizational changes reveal the responsive organizational theory in action, wherein the institutions adjust their structures to meet the new challenges in the environment without altering their central mission of assisting youth development. Media literacy as an addition to mental health programming can be viewed as a paradigmatic shift in the approach towards empowerment-focused interventions as opposed to the traditional deficit-based programming that places young women in the role of active agents instead of the passive consumers of digitally distributed content.

This practice is aligned with the conceptualization of rights of children in digital spaces presented by Livingstone and Third (2017), the tension between the right to protection and the right to participation as organizations negotiate the delicate balance between providing youth with the skills of critical analysis and considering the vulnerability of children to harmful content. The intentional instruction of message production, source criticism, and intended effect analysis demonstrates an advanced take on digital literacy as a safeguard against misinformation, AI-generated content, and curated unrealistic representations that affect young women disproportionately in self-perception and mental health (Tufail et al., 2024). Organizations are emphasizing the education of young people on leveraging social media as a resource and managing its use to mitigate adverse effects. This approach acknowledges that completely avoiding digital platforms is both unrealistic and undesirable for adolescents, whose social and academic lives are significantly influenced by technology (Purnama & Asdlori, 2023)

Navigating Resource Constraints

The results inform the resource dependency theory intricacies in nonprofit organizations working with BBFTPTs, and how funding limitations are an underlying determinant of programmatic capacity and strategic decision-making. Funding realities also had a direct impact on geographic reach, program focus, and service delivery models, with organizations consistently showing an understanding of the precarious nature of balancing Mission preservation and donor compliance. This conflict echoes the main tenets of resource dependency theory, whereby the outsider resource controllers profoundly influence organizational priorities and functioning capacities (Guo, 2007). The explicit recognition by participants that program form changes in response to funder priorities but that the therapeutic goals remain the same exemplifies what scholars describe as strategic adaptation within resource limitation. Some of

the mitigation strategies employed by organizations to maintain autonomy entailed the development of pilot programs that did not have start-up funding, innovative uses of volunteers, and programming models that could be adapted to fit various funding sources. However, the ongoing problem of staffing and technology disparities and access to specialized instruments highlights the inherent constraints of resource scarcity. These limitations are indicative of wider processes of marketization in the nonprofit sector, whereby organizations are being asked to provide innovative services within increasingly limited circumstances and remain accountable to a variety of stakeholder groups whose interests may conflict.

The resultant formation of strategic partnerships amongst the various participating organizations is an advanced institutional adaptation to the constraints of resources and difficulties in service provision. Organizations have strategically built collaborative networks that have gone beyond basic sharing of resources to include sharing of knowledge, professional learning, and problem-solving efforts. These arrangements play several important roles, such as tapping into specialized clinical skills, expanding the geographic range of service, and generating economies of scale beyond those that could be attained by separate organizations. The strategic orientations of such engagements are in line with the thoughts of institutional theory, which suggests that organizations adapt to environmental pressures by engaging in legitimacy-building actions (Stone & Ostrower, 2007). The participants repeatedly mentioned collaborations with mental health practitioners, academic institutions, and peer organizations to provide a broad spectrum of services, especially in light of the multifaceted integration of social media effects and mental health issues among the target demographic. The collaborative strategies manifest organizational awareness that multifaceted mental health challenges need interdisciplinary skills and convergence of resources. Moreover, such alliances also serve as legitimacy-creating tools,

assisting organizations to build credibility among funders, community stakeholders, and prospective participants.

The dominance of grant mandates upon program design indicates severe conflicts between needs as defined by communities and priorities as dictated by funders, pointing to inherent problems with the resource-reliant service delivery modes. Organizations must negotiate a complicated path between program integrity and responding to particular grant requirements, frequently leading to programmatic adaptations that are not perfectly aligned with the needs of participants or the expertise of the organization. This relationship is an example of the main assumptions of resource dependency theory, which states that external financiers have significant control over the organizational decision-making process and strategic focus (Guo, 2007). The geographic and programmatic limitations of the funding sources specifically limit the organizations in creating holistic, culturally responsive interventions to deal with the effects of social media on Black and Brown female youth. Participants repeatedly voiced their dissatisfaction with a funding system that does not acknowledge the cross-cutting nature of mental health issues and the importance of flexible, locally-based strategies. Organizations respond by initiating innovative adaptation approaches, such as diversifying funding portfolios, creating unfunded pilot programs to prove efficacy, and championing community-generated research, which is more reflective of participant voices and needs. These adaptations reflect organizational resiliency and dedication to mission-based service provision amidst structural limitations.

The resource dependency problems observed within various organizations raise urgent questions on the effectiveness of nonprofit governance and the ability to represent the community in a limited funding context. The results indicate that a high dependency on

government and foundation grants could undermine the capacity of organizations to retain robust, representative governance arrangements that accurately mirror the interests and priorities of the community. This strain is also in line with findings by Guo (2007) that government funding dependency reduces the chances of building robust, representative boards that act as true representatives of the community interests, as opposed to being funder compliance tools. These ongoing funding pressures compel organizations to engage in reactive as opposed to proactive programming, which may restrict their ability to meet emergent community needs or respond nimbly to fluctuating conditions impacting their target demographic. The reliance on grant-based programming metrics can foster unintentional organizational focus on community-defined results to funder-designated deliverables, which can ultimately cause damage to the democratic role of nonprofit organizations as representatives of the community. Nevertheless, the strategic partnerships and collaborative practices recorded hold possible clues to how the community may keep itself accountable amid the process of resource limitation.

Institutional Alignment and Legitimacy

The results shed light on how BBFTPTs serving organizations can maneuver through complicated institutional demands without losing their anchor of helping combat the negative effects of social media on mental health. In line with the focus of institutional theory on legitimacy-building processes (Scott, 2001), each of the seven participating organizations exemplified a strategic fit with recognized frameworks like CASEL and trauma-informed practices, and at the same time practiced what Scott (2001) calls decoupling: formal adherence to institutional demands and at the same time retaining culturally authentic programming based on community knowledge. This doubled strategy is an advanced institutional resistance challenge to customary isomorphic strains in the youth development program. The unwillingness of the

organizations to make fundamental changes to their core programming as a way of complying with the wishes of the funders illustrates how the agencies can manage the regulatory space without sacrificing their aim of serving marginalized populations. The discussion of fragmented resources in regulatory spaces offered by Scott (2001) is an important context for considering how these organizations use informal forms of power, based on community knowledge and cultural authenticity, to avoid full institutional conformity.

The uniform focus on culturally relevant programming amongst the participants constitutes a radical shift in the traditional institutional conceptualization of adolescent mental health services, explicitly contradicting what Gay (2018) describes as the inability of mainstream frameworks to accommodate the particular experiences of marginalized groups. This institutional innovation takes the form of identity-based programming that directly challenges damaging social media narratives by allowing youth to reclaim their narratives and subvert stereotypical portrayals. The testimonies of participants illuminated how culturally specific practices are strategies of institutional accommodation required to surmount the structural impediments to effective service delivery, specifically concerning the mislabeling of the communicative styles and mental health demands of Black and Brown girls. Introducing peer support models, community-based celebrations, and identity-affirming practices constitutes what Ladson-Billings (2014) refers to as culturally sustaining pedagogy methods, which not only recognize the cultural identity of the students but also go deeper to maintain and enhance it. The findings build on the literature by Gay (2013) on culturally responsive teaching and how cultural relevance serves as a form of institutional resistance to establish alternative frameworks to conceptualize and approach mental health struggles.

The data demonstrates the complex processes of how organizations build and sustain institutional legitimacy and disrupt the existing traditional approaches to mental health among Black and Brown female youth. Participants reported the adoption of responsive curriculum development procedures that emphasize community voice or choice rather than external directives as a means of developing what Scott (2001) terms as alternative forms of accountability that lie beyond formal regulation frameworks. This legitimacy-building strategy poses a direct challenge to hierarchical notions of program development by establishing community knowledge as a legitimate alternative to academic or clinical knowledge.

The focus of the organizations on evidence-based practices produced through the views of participants as opposed to outside research illustrates an epistemological transformation that recognizes the validity of community-based knowledge creation without sacrificing its legitimacy within the walls of institutionally interested parties. The collaborative partnership became a key solution to addressing resource constraints and maintaining cultural relevancy, generating networked forms of service delivery that disseminate authority among the various service delivery actors within the regulatory space. These results also confirm the conclusion by Scott (2001) that effective regulatory relationships are defined not by independence but rather interdependence as organizations utilize partnerships as a means to deliver full-service offerings without having to sacrifice the primary cultural commitments of the organization.

Curriculum as Adaptive Innovation

The results support the premise that nonprofit organizations working with BBFTPTs can be viewed as active drivers of curricular development, as opposed to passive deliverers of a pre-determined mental health program. This dynamic ability is consistent with the strategic responsiveness and stakeholder-oriented refinement of change management theory (Akingbola et

al., 2019). Organizations demonstrated incredible adaptability in how their programming evolved after COVID-19, developing new staff roles (mental health counselors) and incorporating elements of media literacy, as well as expanding existing support networks to caregivers and school staff. These strategic accommodations are reflective of what Durlak and DuPre (2008) conceptualize as "implementation fidelity with flexibility," whereby essential therapeutic outcomes are maintained, but the delivery models remain adaptive to participant contexts and emergent needs. The feedback systems outlined systematically throughout the organization, ranging from formal surveys to informal chats, highlight the cyclical process of good program development. Such responsiveness calls into question more traditional top-down models of programming and places youth voice at the center of curriculum design, aligning with organizational theory, which highlights adaptive capacity as a key factor in survival in dynamic settings (Akingbola et al., 2019).

As opposed to the dominating deficit-driven discourse, the involved organizations reconceptualized social media as a platform with multidimensional potential, empowering youth and enabling them to rebuild their narratives. This re-framing is in line with youth participatory action research ideas that assert the potential of young people as young agents of social criticism and social healing via storytelling. Workshops and programs that featured podcasting, visual storytelling, and spoken word exercises offered girls not only analytic frameworks to understand how to break down the damaging messages they encountered online but also creative outlets to generate counter-narratives that reflected their identities and experiences. Such interventions fulfill the ideal of education as the practice of freedom that Cannon (2016) describes as the critical inquiry transformed into the act of liberation, as opposed to an academic exercise. Implementing media literacy as a mental health intervention would be a paradigm change, a shift

towards a structural analysis of oppressive systems rather than an individual pathology model. Organizations recognized that by providing the youth with essential media literacy skills, they could ensure that they were able to traverse the digital landscapes as empowered consumers and creators instead of being passive recipients of potentially harmful content.

The community-building and peer support focus in all of the participating organizations indicates a basic recognition that healing is a collective, not an individual, process. This practice disrupts conventional mental health service provision models because it does not prioritize clinical treatment patterns but instead emphasizes relational healing and co-experience. Institutions hosted new practices like healing circles, ambassador programs, and intergenerational therapy sessions that made mental health conversations ordinary while decreasing stigma because of shared vulnerability. These community-based methods also adhere to youth participatory action research paradigms, which embrace the collective wisdom and the power of peers in influencing social change. The establishment of a variety of programming choices between intensive dialogue sessions and arts and crafts activities shows that the organization will go the extra mile to accommodate different learning styles and activity preferences. This multi-modal adaptation acknowledges the fact that effective program implementation must be able to adapt to the needs and contexts of participants (Durlak & DuPre, 2008). The combination of creative expression in the form of meditation, journaling, and artistic activities offers accessible points of entry into working on more complex emotional issues and the development of necessary coping skills.

Although innovation and effectiveness were demonstrated, the participating organizations consistently reported funding limitations and institutional barriers as major constraints to the complete program implementation and sustainability. This tension informs the greater structural

limitations to community-based organizations seeking to offer culturally responsive mental health interventions in systems built on a deficit-based model. The attempts of organizations to translate language and frameworks in the transition between a pathologizing discourse to a strength-based model constitute what the institutional theory describes as resistance to isomorphic pressures, which preserves inefficient status quo practices (Akingbola et al., 2019). The call to policy-level interventions reported by participants can be understood as an acknowledgement that individual behavior change is not enough to combat the systemic factors that contribute to mental health struggles among Black and Brown youth. This view is similar to the criticism presented by Cannon (2016) against mindfulness education, which only teaches individual self-regulation but does not mitigate structural oppression.

Connection to Research Question

The findings of the study illustrate how organizations have transcended traditional medical frameworks to adopt a culturally responsive approach that considers the distinct intersectional issues affecting BBFTPTs in the digital realm. The realization that condensing the actions of these young women to being spicy or having attitudes takes away any chance of empathy with their psychological lives is the mark of a nuanced grasp of how cultural stigma versus mental health within Black communities intersects with the issues of the digital age. The use of healing circles, storytelling, and peer support groups within organizations reflects a combination of culturally appropriate therapeutic processes that acknowledge the healing traditions of the community but also deal with the stressors of the digital age. Such a move is consistent with the growing body of research on digital resilience, which argues that cultural context is crucial to how youth negotiate the online environment (Livingstone & Third, 2017). The results indicate that any effective mental health promotion strategies in this group need to

work concurrently on the external sources of the digital vulnerability, such as racism, body shaming, stereotyping on social media, and developing internal resources of critical engagement with media and constructing a positive identity. The observations regarding consistent funding issues outlined throughout the organization point to the fact that the conceptualization, funding, and evaluation of mental health services need systematic change, especially when it comes to marginalized populations whose needs are typically misconstrued or poorly met within the context of the traditional delivery of services.

These findings hold critical implications related to the potential effects of culturally sensitive mental health programming to intervene in the effects of social media on Black and Brown female adolescents without compromising their navigation through complicated institutional landscapes. The ability of the participants to preserve cultural authenticity and attain institutional legitimacy implies that perhaps the traditional distinctions between community-based and institutionally sanctioned modes of operation are false constructs that constrain programmatic innovations. This focus on media literacy and narrative reclamation as central to mental health programming is an advanced recognition of how digital spaces collide with identity formation in marginalized youth and goes beyond a strictly clinical conception of mental health to embrace the ideas of critical pedagogy. The success of these organizations in resisting institutional isomorphism and remaining operationally viable illustrates the promise of culturally sustaining practices to alter larger youth development and mental health service delivery systems. The study informs currently debated discussions on decolonizing mental health practices by showing how organizations can effectively avoid institutional forces driving towards uniformity and instead develop effective, culturally rooted interventions that resonate with the complexities of teenage life in online environments.

Table 12

Contributions of Findings to the Research Question and Theoretical Frameworks

Analysis Theme	Key Findings	Relevant Theoretical Framework(s)	Contributions to Research and Theory
Responding to the Social Media Landscape	Identified a rise in internalized mental health symptoms post-COVID driven by social media; organizations implemented media literacy and trauma-informed curricula.	CMT; IT	It shows how nonprofits adapt to emerging digital risks while balancing institutional expectations and preserving cultural authenticity.
Navigating Resource Constraints	Organizations faced chronic funding instability, technology access issues, and staffing limitations. They responded through collaborative partnerships, volunteer-led programming, and grant compliance strategies while maintaining core therapeutic goals.	IT; RDT	Highlights strategic adaptation under resource pressures and how collaborations build legitimacy while sustaining culturally relevant programs.
Institutional Alignment and Legitimacy	Despite regulatory pressures, organizations formally align with CASEL/trauma-informed frameworks while sustaining culturally specific curricula. They resist full isomorphic pressure by privileging community knowledge and culturally sustaining pedagogy in program design.	IT	Illustrates decoupling in action—formal alignment with institutional norms alongside culturally grounded resistance and alternative accountability.
Curriculum as Adaptive Innovation	Curricula centered on youth voice, community feedback, and creative expression (e.g., podcasting, storytelling, healing circles). Programs emphasized relational healing, media literacy, and identity formation, diverging from clinical norms.	CMT; IT	Demonstrates flexible, community-led curriculum design and resistance to top-down models, advancing adaptive and culturally sustaining approaches to mental health education.

[*] Resource dependency theory (RDT

[**] Institutional theory

[***] Change management theory

Implications for Practice

The findings of this study carry significant implications for nonprofit practitioners, mental health educators, and organizational leaders designing programs for Black and Brown female youth in today's digitally saturated environment. As organizations grapple with rising mental health challenges linked to social media use, the study highlights several actionable practices that can enhance the responsiveness, cultural relevance, and sustainability of nonprofit programming.

Integrating Media Literacy as Mental Health Strategy

Given the pervasive influence of social media on adolescent well-being, media literacy should be treated as a core component of mental health education rather than a supplemental skill. Nonprofits should invest in training staff on how to facilitate nuanced conversations around curated content, misinformation, online identity, and the emotional effects of algorithm-driven platforms. As Theodorio (2024) observed, educators and practitioners should be equipped with both the pedagogical tools and the technological resources to help youth navigate the digital world safely and reflectively. Programs that allow Black and Brown girls to critique, reframe, and reclaim narratives from social media—through storytelling, spoken word, podcasting, or visual media—foster both emotional resilience and critical consciousness, as suggested by Livingstone and Third (2017). This sentiment is also upheld by leading scholars in the fields of media and mental health such as Burke and McKenna (2018) and Jimenez et al. (2022). Nonprofits should formalize inclusive, iterative curriculum development processes that are informed by data, beneficiary feedback, and cultural knowledge. Regular evaluation and revision cycles, informed by both formative and summative assessments, will ensure that programming remains responsive to changing needs and emerging issues in digital mental health (Bucci et al.,

2019). These results can guide academic institutions in establishing curriculum and research priorities to meet at the nexus of social media, mental health, and racial identity among adolescent populations. These outcomes can also be taken into consideration when schools build a multifaceted strategy towards addressing the mental health concerns of Black and Brown female students regarding exposure to social media.

Centering Cultural Relevance and Identity Affirmation

The study results add to the current body of knowledge about the urgent necessity of culturally responsive mental health programs (Erigha & Crooks-Allen, 2020; Volpe et al., 2021), and indicate that the organizations working with Black and Brown female teenagers and pre-teeners should fundamentally reorganize their efforts to target the complex relationship between social media and mental health. These emerging themes solidify the notion that conventional mental health paradigms are inadequate when seeking to serve the intersectional needs of this demographic (Erigha & Crooks-Allen, 2020). Nonprofit organizations should continue to develop curricula that are not only trauma-informed but also culturally responsive and identity-affirming.

The study firmly suggests that the design of the program should be driven by the youth in the sense that all intervention measures should be centered on the voices of participants in the curriculum development and delivery processes. Organizations must focus efforts on establishing identity-driven programming that empowers cultural relevance and integrates innovative mental health strategies, including media literacy education, storytelling, and peer support systems. Moreover, the organizations should invest in an organization-wide training to build cultural competency and adopt adaptive language models that shift the focus beyond deficit-based models to a strength-based model that recognizes the lived experience and resilience of Black

and Brown female youth (Urgun et al., 2025). In alignment with the earlier studies, authors in later studies suggest that practitioners should prioritize youth-centered design processes that allow girls to co-create content, reflect on their lived experiences, and find strength in community representation (Frigha & Crooks-Allen, 2020; Garcia, 2020). This is strongly supported in the literature, indicating that community-based organizations must focus on strategic partnerships and resource sharing to help them transcend individual organizational shortcomings while preserving cultural authenticity in service delivery (Agonafer et al., 2021).

Nonprofit organizations can extend their reach and deepen impact by forging partnerships with schools, healthcare providers, arts organizations, and family networks. Partnerships can help mitigate resource constraints and enhance credibility when implementing curriculum within institutional settings (Amey & Eddy, 2023). Practitioners should view schools not just as sites for program delivery, but as co-educators and allies in promoting digital wellness and emotional development among youth. Schools need to build partnerships with community-based organizations specialized in working with Black and Brown youth, developing referral networks to get students connected with culturally competent mental health supports. A media literacy curriculum can be incorporated into educational institutions and specifically focus on the intersectional issues that students of color are exposed to, such as online racial discrimination, colorism, and unrealistic beauty standards (Mady et al., 2023). Universities need to form collaborations with community groups to do research that is produced through the lens of subjects and community members as opposed to researcher-imposed structures and agendas. Social work, psychology, and public health academic programs can incorporate coursework on digital media literacy, cultural responsiveness, and intersectional mental health interventions (LeBlanc & Loyd, 2022). Moreover, these establishments should also focus on educating a

variety of mental health workers capable of helping marginalized groups and promoting the development of more research studies that would explore the peculiar issues youth of color may struggle with within the digital environment.

Advocating Flexible Funding Models

One of the most critical implications for practice centers on how nonprofit organizations can maintain the integrity and cultural relevance of their curricula while operating within the constraints of donor expectations and grant requirements. The findings of this study reveal that while many organizations are deeply committed to youth-informed and equity-driven programming, inflexible funding structures often restrict their ability to innovate and respond to emerging needs (Williams, 2023). To safeguard their capacity to deliver responsive mental health curricula—particularly those that address the unique digital challenges facing lack and brown girls—nonprofits must advocate for flexible funding models that support adaptation, experimentation, and long-term impact rather than narrowly defined outputs.

According to Mumford (2022), nonprofit leaders should also prioritize strategic communication with funders, emphasizing the importance of holistic, trauma-informed, and culturally grounded programming. Practitioners must develop the capacity to clearly articulate how long-term community wellness and youth empowerment may not always align with short-term, easily quantifiable metrics (Mumford, 2022). Cultivating relationships with funders who share an equity-oriented mission and who value community voice can support more authentic, mission-aligned programming and reduce the need to compromise on core values.

The successful implementation of curricula addressing the mental health effects of social media also depends heavily on well-trained and adequately supported staff. The study findings underscore the need for ongoing professional development in areas such as digital trauma,

adolescent development, cultural humility, and creative therapeutic practices (Ranjbar et al., 2020). Moreover, organizations must budget for technological and artistic resources—such as video/audio editing tools, tablets, internet access, and creative supplies—that enable participants to engage in healing-centered storytelling, digital critique, and self-expression. Without these investments, programs risk becoming under-resourced and ineffective, especially in communities where digital access is already unequal (Raihan et al., 2025).

At the policy level, these findings suggest an urgent need to reexamine existing funding structures and public grant mechanisms, which often reflect narrow, prescriptive program models that fail to account for the complex and intersecting needs of marginalized youth. Funding models that favor discrete deliverables over systemic outcomes limit the ability of organizations to address issues such as digital identity, online harassment, intergenerational trauma, and racialized media content (Quinn, 2025). Policymakers should develop and expand funding streams that explicitly support culturally responsive, media-literate, and post-pandemic mental health initiatives.

Furthermore, this study highlights the need for structural reforms that prioritize community-based research, allow for unrestricted or semi-restricted funding, and recognize the expertise of frontline practitioners and affected communities. In tandem, digital policy reforms are necessary to mitigate the mental health risks posed by online environments. Koehle et al. (2022) states that policymakers should work to combat online racial discrimination, enforce digital safety standards, and protect vulnerable youth from harmful algorithmic content that disproportionately affects Black and Brown girls. Aligning mental health funding and digital safety policy with community needs and lived experience is essential for building sustainable, equity-driven systems of care.

Advancing Community-Based and Intergenerational Healing Models

Another important implication of this study lies in its relevance to community-based mental health interventions that move beyond traditional clinical models to address the broader social determinants of mental health (Hamilton et al., 2022). The findings suggest that to meaningfully support Black and Brown female adolescents affected by the harmful aspects of social media, communities must cultivate extensive, culturally grounded support systems that respond to post-pandemic mental health realities.

The study confirms the claims that peer support models are especially effective with this group (see Mance-Early et al., 2024), thus recommending the integration of mentorship programs and healing circles in schools that provide safe environments where Black and Brown female students can express themselves genuinely and build community. This involves investing in intergenerational healing initiatives that acknowledge both historical trauma and contemporary stressors, creating spaces where mothers, daughters, caregivers, and extended kin networks can engage in collective healing journeys (Cole et al., 2025). Such programs not only build resilience among youth but also foster communal solidarity and emotional literacy across generations.
In addition, practitioners and community leaders should advocate for increased mental health resource allocation at local and state levels, especially in under-resourced communities, while actively working to destigmatize mental health help-seeking among black and Black and Brown populations (Castillo et al., 2019). Community-wide education campaigns, peer support networks, and partnerships with faith-based and grassroots organizations can help normalize conversations about mental well-being and digital stress, thereby empowering young women to access culturally responsive and affirming care that reflects their lived experiences in both physical and digital spaces.

Strengths and Limitations

The main strength of this study lies in its focus on a critical and underexplored issue: how community-based organizations are addressing the mental health impact of social media on BBFTPTs. While there is growing recognition of social media's effects on youth mental health, few studies have examined how frontline nonprofits respond to these challenges through curriculum design. By centering the voices of nonprofit leaders serving marginalized youth, the study fills a critical gap in both education and mental health research. The study also demonstrates a commitment to equity and cultural responsiveness by highlighting how these organizations center the voices, needs, and identities of Black and Brown girls. This ethical and scholarly positioning affirms the role of nonprofits as key agents of social change and mental health advocacy within historically underserved communities. The study's practical relevance and applied orientation strengthen its contribution to the field.

Another notable strength of the study lies in its theoretical foundation, which is anchored in three complementary frameworks: resource dependency theory, institutional theory, and change management theory. This combination offers a comprehensive perspective for understanding how nonprofit organizations make curriculum decisions in complex environments. By applying these intersecting frameworks, the research explores not only the impacts of funding and institutional pressures but also the internal dynamics of organizational adaptation and strategic planning. This triangulation of theories enhances the clarity of the findings and adds valuable insights to the existing literature on decision-making and organizational behavior within nonprofits, while the concrete examples of best practices, programmatic innovations, and strategic partnerships can inform other nonprofit organizations, funders, educators, and policymakers.

The qualitative methodology used in this study is another strength. Conducting semi-structured interviews with seven nonprofit leaders allowed for in-depth, context-rich insights into the processes and challenges of curriculum development. The diverse organizational sample, with operating experience ranging from 2 to 13 years, offers valuable insights into the various developmental phases of nonprofit organizations, encompassing both start-up initiatives and well-established programs. The geographic concentration on the South and West sides of Chicago allows for a specific focus on historically underserved communities, where mainstream mental health services have often been either limited or culturally misaligned. Additionally, the incorporation of a thematic analysis, guided by the coding framework of Braun and Clarke's multi-case approach, enhances analytic rigor and enables the systematic identification of patterns across the variability of organizational models. This process reveals common themes and meaningful distinctions among organizations of different sizes, structures, and service delivery models. Furthermore, the cross-case matrix structured coding approach used in Microsoft Excel facilitated comparative analysis and could serve as a potential tool for replication or further research.

While this study provides valuable insights into how nonprofit organizations design mental health training curricula to address the impact of social media on Black and Brown girls, several limitations should be acknowledged. First, the small, purposive sample of seven organizations, all based in Illinois, limits the generalizability of the findings. Although the goal of qualitative research is not statistical generalization, the regional concentration of the sample may not fully capture the diversity of nonprofit practices across other states or rural versus urban contexts. Nonprofit organizations operating in different policy environments or with different funding landscapes may face distinct challenges and opportunities that were not reflected in this

study. Second, the reliance on interviews with a single representative from each organization may have constrained the depth and breadth of organizational perspectives. While participants were in leadership roles and knowledgeable about curriculum development, they may not have fully represented the views of program staff, mental health practitioners, or youth participants themselves. Future research incorporating multiple stakeholders, including direct service providers and program beneficiaries, could offer a more comprehensive understanding of how curriculum decisions are made and experienced.

Given that the study used self-reported data, responses may have been influenced by social desirability bias or participants' intentions to portray their organizations in a positive light because the study relied on self-reported data. While probing questions were used to elicit examples and clarify responses, the absence of observational or documentary data means that claims about implementation practices or program impact could not be independently verified. Lastly, the application of the theoretical framework, resource dependency theory, institutional theory, and change management theory was guided by the researcher's interpretation. While these frameworks provided valuable analytic depth, alternative or complementary theories (e.g., critical race theory, feminist pedagogy, or ecological systems theory) might highlight different dimensions of organizational decision-making, particularly those related to power, identity, or structural inequality. Despite these limitations, the study offers rich, contextually grounded insights that contribute to a deeper understanding of how nonprofits innovate in response to pressing mental health and social issues affecting Black and Brown youth.

Future Research Directions

Building on the comprehensive findings of this study, which explored how Illinois-based nonprofit organizations design and implement mental health training programs for BBFTPTs,

several critical avenues for future research emerge. This study's identification of eleven thematic areas provides a strong foundation for expanding inquiry into both programmatic effectiveness and the structural forces that shape service delivery. Future research should prioritize longitudinal studies that assess the sustained impact of media literacy interventions and identity-based programming on youth outcomes, including resilience, self-efficacy, emotional regulation, and digital citizenship. Such research would help establish evidence of long-term benefits and clarify the developmental trajectories influenced by culturally responsive curricula.

Comparative studies are also warranted to explore whether the approaches identified in this study translate effectively across demographic groups, including boys, gender-diverse youth, and youth of different racial and ethnic backgrounds. Investigating how gender identity and cultural context intersect in youth mental health programming could reveal whether the emphasis on cultural affirmation, narrative agency, and community-based support is universally effective or requires adaptation to meet the needs of diverse populations. These studies could also examine whether identity-based pedagogies function differently based on gender socialization and digital engagement patterns.

The study's findings regarding institutional pressures and resource dependencies further suggest important directions for policy-oriented research. Specifically, there is a need to examine how funding structures—from private philanthropy to government grants—influence program design, implementation fidelity, and the sustainability of culturally responsive practices. Future research should investigate the relationship between financial flexibility, funder accountability, and mission integrity, especially in organizations serving marginalized populations. Additionally, analysis of the policy environments that govern curriculum standards and digital education could

illuminate how federal, state, and local regulations either constrain or enable nonprofits' capacity to innovate in response to emerging mental health challenges.

Research exploring the scalability and transferability of effective program elements—such as trauma-informed media literacy, narrative reframing, and family-inclusive programming—would also be valuable to policymakers and practitioners seeking to replicate or expand successful interventions. These investigations could inform the development of toolkits, best practice models, and funding mechanisms designed to promote widespread adoption of effective curricula.

Finally, future studies should explore the intersections of digital rights, mental health equity, and community-based service delivery. As nonprofits increasingly confront the challenges of misinformation, online trauma exposure, and algorithmic bias, research that addresses how youth-serving organizations can navigate these risks without undermining the digital agency of vulnerable adolescents will be essential. Such studies could contribute to the development of policy frameworks that center both youth protection and empowerment in digital spaces, ultimately leading to more inclusive, responsive, and sustainable mental health support systems for Black and Brown girls and other historically marginalized youth.

Final Conclusions

This study contributes to a deeper understanding of how Illinois-based nonprofits respond to the complex mental health needs of BBFTPTs in a post-pandemic, digitally saturated world. The study employed a qualitative methodology involving seven nonprofit executive interviews and examined organizational strategies through the lenses of resource dependency theory, institutional theory, and change management theory. Nonprofit organizations in this study demonstrated remarkable agility in managing institutional pressures while responding to shifting

youth needs and embedding media literacy into mental health programming. Despite ongoing challenges related to funding constraints, staff capacity, and the evolving nature of digital misinformation, these organizations exemplify mission-driven innovation while preserving culturally responsive programming. leveraging community partnerships. Their ability to balance institutional alignment with autonomy and to translate trauma-informed and culturally responsive practices into curriculum that is meaningful and transformative for youth offers a valuable model for others in the field.

This study adds to the existing literature on culturally responsive mental health programs by highlighting the need to ensure youth-driven program modeling and community-based program implementation that challenge deficit-based program models toward strength-based program models. The results reveal the extent to which organizations effectively resist institutional isomorphism by adopting novel approaches to stress management, including healing circles, storytelling workshops, and peer support networks that respect and embrace the healing traditions of the community and yet respond to the stress factors of the digital age.

Organizations indicated the necessity of strategic collaborations and sharing of resources to transcend the limitations of individual organizations, without losing the cultural authenticity in the delivery of services. Inclusion of media literacy as a mental health intervention is a paradigmatic change, placing young women in the role of agents instead of passive receivers of the digital content, recognizing their potential for digital resilience and creativity. Future studies may further extend the sample to rural or suburban organizations, compare programs serving boys or gender-diverse youth, or longitudinal outcomes of media-based mental health programs to better comprehend program efficacy and sustainability.

For nonprofit leaders, this study underscores the importance of investing in staff development, co-designing with youth, and pursuing funder relationships that preserve curricular integrity. For policymakers and funders, the findings call for greater flexibility in grant structures, increased investment in mental health infrastructure, and policy frameworks that recognize the role of community-based organizations as frontline innovators in youth mental health and digital wellbeing.

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Appendix A: Interview Protocol

Interview Questions

1. Can you describe the process your organization follows to develop the curriculum for mental health training programs?
2. What factors or considerations are most influential in shaping (1) the content and (2) implementation of your mental health training programs?
3. Before we proceed to the specific questions, could you please share anything about your nonprofit and its mission services or beneficiaries? #staff, #beneficieres, #locations, #partners, #years, #grants, #donors for the service?
4. How have the mental health needs of Black and Brown female teens and preteens changed in your experience before and after the COVID-19 pandemic?
    a. What specific (mental health) challenges have emerged or become more pronounced since the pandemic?
5. How has your organization adapted its mental health services to address the changes in the needs of Black and Brown female teens and preteens post-pandemic?
    a. Have any new strategies or program elements been introduced in response to these changes?
6. How does your organization respond to the specific mental health needs of Black and Brown female teens and preteens?
    a. How does your organization identify the needs?
7. Can you describe the process your organization follows to develop the curriculum for mental health training programs?

8. What factors or considerations are most influential in shaping (1) the content and (2) implementation of your mental health training programs?
9. How does your organization address the negative effects of social media on mental health in your training programs?
    a. Can you provide examples of specific materials, activities, or strategies used to help Black and Brown teens and preteens navigate these challenges?
    b. Have these factors impacted the inclusion of social media-related topics?
10. What challenges does your organization face in integrating social media-related topics into mental health training curricula?
11. Are there any opportunities or best practices you've identified that could enhance these programs in the future? What would your organization need to make it happen?
12. If there is anything else, please share.

Appendix B: Invitation Letter

Subject: Invitation to Participate in a Research Study on Mental Health Training Programs

Dear Recipient,

I hope this message finds you well. My name is Darryca Brim Mims, and I am a doctoral student at Governors State University (GovState) in Illinois. I am conducting **a research study titled** *"Exploring the Development of Mental Health Training Programs for Black and Brown Female Teens and Preteens in Illinois Nonprofits."* The research is supervised by Assistant Professor Dr. Erica Ceka.

The purpose of this study is to explore the mental health services currently delivered by IL nonprofits and explain the context and the conditions under which nonprofits could focus their training curriculum to ensure it addresses the negative impacts of social media on the mental health of Black and Brown female teens and pre-teens. This research aims to provide valuable insights and recommendations for enhancing mental health training programs in nonprofit organizations.

We are seeking participants who meet the following **inclusion criteria**:

Currently employed by an Illinois-based nonprofit organization offering mental health training programs for Black and Brown female teens and preteens.

Hold an executive-level position (e.g., CEO, program director) with professional knowledge of service delivery, curriculum development, and decision-making processes within the organization.

Participation involves **a single interview via Zoom, which lasts approximately 60 minutes**, to discuss your organization's mental health training programs and the factors influencing their design. You will **be asked open-ended questions** related to your professional experience developing and modifying mental health training programs, **such as** "Can you describe the process of developing a curriculum for mental health training?" Your basic demographic information will also be collected.

Participation in the study is voluntary, and you may refuse to participate or exit the study at any time. You may also skip any questions you do not want to answer.

No incentive is being offered for your participation other than gratitude for helping us address known research gaps to potentially inform evidence-based practice for helping this group mitigate the negative effects of social media usage.

The risk of participation in this study is minimal. However, you should be aware that there is always the possibility of security breaches of data collected via the Internet. **All responses will be kept confidential**, and we will make every effort to protect your responses. After analyzing the responses, **the data will be de-identified and stored on a secure server** at GovState, and it will not be shared with anyone outside the research team.

The study has been reviewed and approved by the GovState Institutional Review Board (IRB). If you have any questions about this survey, please contact Darryca Brim Mims (dbrim@focusfairiesmentoring.org) or Dr. Erica Cela (eceka@govst.edu). If you have any questions regarding your rights as a research participant, you may contact the GovState IRB at 708-235-2147 or email at irb@govst.edu.

If you are interested in participating, please reply back at dbrim@focusfairiesmentoring.org.

Thank you for your important contribution to this study!

-Darryca Brim Mims

Please print or save a copy of this invitation for your records.

Appendix C: Verbal Consent Script

Thank you for joining us today for the interview.

Last week, we emailed you an information sheet about the purpose of the study and this interview. Please keep the information sheet for your reference.

To remind you, the purpose of this interview is to explore your experience as a nonprofit professional in development and delivery of mental health training programs for Black and Brown female teens and preteens, including the integration of materials and activities addressing the impact of social media on mental health, and factors influencing curriculum development and decision-making processes within your organizations. Participation in this discussion is completely voluntary, and you may stop participating without any penalty. The duration of the interview is about 45-60 minutes.

The interview will be audio-recorded and then transcribed. The transcriptions will be de-identified to protect your confidentiality. Also, we will keep your email addresses until the end of the study. We may contact you to check our interpretation of what was said in the interview to ensure we got things right. If you don't want to be contacted, please email me (Darryca Brim Mimms) at dbrim@focusfairiesmentoring.org or Dr. Erica Ceka, faculty advisor, at eceka@govst.edu.

The findings from this interview will be used for academic publications and to inform public policies.

Do you have any questions for us?

Do you agree to participate?

Appendix D: Participants' Bios

This appendix provides a comprehensive overview of the short biographies for each participant (referred to as P in the narrative) who was interviewed as part of this study. Each biography is crafted using deidentified pronouns to maintain confidentiality and ensure privacy. The narratives highlight the unique backgrounds, experiences, and perspectives of the participants, offering valuable context for understanding their contributions to the research.

Participant 1: P1 is the founder of a nonprofit organization started in October 2018 in Illinois. Having been in practice for seven years, they have specialized in working with families and youths, specifically in literacy and development for children aged between six months and eighteen years. This organization also works within the community, providing art and writing workshops, storytelling, a food pantry, and other community services. They enjoy advocating for BBFTPTs to celebrate and embrace themselves through safe platforms for artistic, mental, and emotional expression. They are also most concerned with responsive programming, which considers the stakeholders' needs in times of implementation with a focus on the social and emotional aspects that affect the learning and development of children.

Participant 2: P2 is the leader of a non-profit organization established in 2017 that aimed at empowering BBFTPTs between the ages of 12–19 living in inner-city areas of Illinois. The organization currently has three staff members and operates in four schools. It's key objective is to connect the link between socio-emotional development and academic performance. It takes a comprehensive approach to college and career readiness, community service, welfare, and socio-emotional development. This organization is based on a cohort model that follows the girls through their high school years, focusing on the development of positive relationships and issues affecting young BBFTPTs.

Participant 3: P3 is the founder of a nonprofit organization that was founded in 2012 and is based in Illinois. Participant 3 organization has been serving for 13 years, and its mission is to address the media representations of BBFTPTs through media literacy, creative writing, and counseling. Currently, the organization targets about 35 girls for direct assistance and impacts between 600 girls within a year through events. Using culturally appropriate strategies, participant 3, who recently completed a master's degree in social work, designed new programs that incorporate training into mental health, circles, media-based, and media-focused sessions for young girls and their families.

Participant 4: P4 is the founder and the executive director of a not-for-profit organization that seeks to support BBFTPTs in Illinois. The program was founded in 2016 and became an official non-profit organization in 2021. It aims to provide comprehensive support for girls in 4th through 8th grade, located in the western and southern parts of Illinois. As a former teacher, participant 4 has transitioned the organization from a combination of occasional voluntary work within the community to a program that enrolls 75-100 girls every year. They have one staff member employed full-time (participant 4) and five part-time employees, and the funding for the organization mostly comes from foundation grants. The organization's mission focuses on empowering girls to be more than what's simply expected, with a focus on leadership, community involvement, fitness, and financial competence.

Participant 5: P5 is the CEO of a local, Illinois affiliate of a national nonprofit that was founded in 2017 with the mission of empowering African American and Latina girls. Participant 5 has been directing the organization for six years now, and the main goals of the organization are to offer effective programming to approximately 1,000 girls annually, from the age of five to college scholarship and workforce development programming. They address mental health,

social issues, and youth development through direct services and interventions, advocacy, and artistic endeavors. As an educator with a passion for empowering girls, Participant 5 has grown the organization's reach from 65 girls at one school to currently serving 8-9 schools while striving to ensure that the program is culturally responsive and relevant to the changing needs of BBFTPTs.

Participant 6: P6 serves as the Program Manager for an Illinois-based nonprofit organization founded in 2014 that focuses on anti-bullying and empowerment for girls ages 8-18. With a background of 13 years in education, P7 brings extensive experience , having been with the organization for one year. The nonprofit operates in Chicago Public Schools and suburban areas, offering both in-school and Saturday community sessions that provide mentoring, social-emotional learning, and community service activities. Currently, the organization serves approximately 75 girls directly and reaches hundreds more through annual events like the Girls Health Matter Summit, with a mission to address mental health challenges and build community among BBFTPTs.

Participant 7: P7 represents a nonprofit organization founded in 2015 that serves BBFTPTs in underserved communities. With a mobile operational model and a small team of approximately three staff members, the organization focuses on holistic health services under a comprehensive umbrella of care. Their mission encompasses mentoring, resources for mothers, annual women's retreats, and mental health support, with an estimated reach of around 100 girls annually. The organization partners with approximately three schools and ten community-based organizations, emphasizing evidence-based approaches and community-driven programming. Their work has increasingly centered on addressing mental health challenges, nutrition, and social media's impact, particularly in the post-pandemic landscape.

Appendix E: Organizational Profile (Contextualization)

ID	Founded	Mission Focus	Target Population /Service scope	Budget Size/Donors	Staff	Affiliate Partnership	Curriculum Focus
P1	2017	To inspire girls to be strong (healthy), smart (educated), and bold (independent)	Serves over 1,000 girls each year	$2.7 million budget; Total revenue: $416,668; Total assets: $28,904	20 paid staff (excluding interns)	Partner with "My CHI. My Future." initiative providing online and mobile hub for youth-centered experiences; Located in Illinois	Holistic development focusing on health, education, and independence; Literacy and reading promotion; Youth empowerment through experiential learning and identity development
P2	2017	To empower minority adolescent girls by bridging the gap between social-emotional learning and academic success through mentorship, college and career exploration, and community engagement	Minority adolescent girls: First cohort consisted of 80 SHE Sisters	Donors include Obama Foundation, Springboard Foundation, Crossroads Fund, ESPN, The Chicago Community Trust, The Chicago Fund for Safe and Peaceful Communities, Chicago Foundation for Women, Leadership	3 staff members including the founder	Instagram @_TeamSHE; Facebook @TeamSHEChicago	SEL Mentorship; Community Engagement; College and Career Exploration; Urban Girls Against Violence campaign

				Mentoring Opportunities Foundation			
P3	2018	To help African American girls critically examine social, cultural, and political ideologies in media to overcome stigmas and negative stereotypes	African-American girls ages 11-17	Total revenue: $514,057; Total assets: $33,546	6 Board members plus Founder	H.O.T Girl Summer camp	Digital storytelling; Media literacy; Community transformation; Global sisterhood
P4	2012	To cultivate empowered individuals who challenge stereotypes, prioritize well-being, embrace unique identities, uplift communities, and sustain lifelong success	Girls in 4th-8th Grade	Sponsors include: Entrenuity, SGA Youth & Family Services, Girls Opportunity Alliance	7 Executive Board Members and 6 leadership members	Partner Organizations include the Gary Comer Youth Center, Breakthrough, Great Lakes Academy Charter School	Leadership Skills; Mentorship; Healthy Relationships; Health & Wellness; Financial Responsibility; Entrepreneurship; Social Responsibility
P5	2017 (Chicago chapter); 1864 (national)	To support girls in developing their strengths, building confidence, and growing up healthy, educated, and independent	Started with 65 girls at Ruggles Elementary; Expanded to 6 additional schools; Served 800	Revenue: $2.35M (2024); Expenses: $2.26M (2024); Total Assets: $1.49M (2024); Total	5 Staff Leadership members and 7 Other Staff	Charles and Lynn Schusterman Family Philanthropies, Lilly Endowment Inc., Macy's, Pivotal	School-based and citywide programs; Focus on skills for lifelong success

			girls in 2021	Liabilities: $559k (2024)		Ventures, Porsche	
P6	2014	To empower girls to prioritize mental health, avoid negativity, build self-confidence, and pursue goals through perseverance, courage, and creativity	Girls ages 8-18; Approximately 230 girls annually, Over 1500+ girls served to date	Budget of 300K, Funding from Oprah Winfrey Foundation and The Girls Opportunity Alliance through the Obama Foundation	4 Board directors and 6 leadership team members	Pacer's National Bullying Prevention Center (2022 CHAMPION, 2023 Unity Award); Illinois Governor J.B. Pritzker (April 5th as Girls Health Matters Day); Village of South Holland (June 1 as M.E.A.N. Girls Day)	Bullying prevention; Mental health; Saturdays with MGE; Tweens Program; Teens Program; MGE University; Project MEAN Streets
P7	2015	Improving the lives of women and girls through education and increased access to resources	Women and girls	Southland RISE (Resilience Initiative to Strengthen and Empower), Share Our Spare	No information provided	Cradles to Crayons, End Clothing Insecurity, Costco Wholesale, The Community	Balancing Mind & Body; Nutritional Wellness; "Why I am Enough" Program focused on faith, mindset development, and personal growth

Appendix F: Excel Coding Framework

Theoretical Alignment	Themes	Definition	Key Words	Examples TEST
RDT* IT**	Organizational Capacity and Program Design	How the organization defines, structures, and delivers its training programs;	Years of operation; mission/focus; budget size; donors; staffing; number of girls served; site locations; partnerships; current curriculum features	Founded in 2017; 3 staff; 4 schools; ~90 girls/year
CMT***	Mental Health Needs Identification	How organizations recognize or assess the mental health needs of Black and Brown girls.	Methods of needs assessment; Student voice and engagement; Changes since COVID-19; increased referrals; reports from parents	Collective breakdown; increased referrals; parental awareness
CMT IT	Curriculum Focus	Educational frameworks, design principles, and intended outcomes of the training curriculum. Strategies used to address the negative influence of social media in the mental health curriculum.	SEL or trauma-informed; content themes; use of CASEL, narrative tools, etc.	CASEL & MHA; focus on SEL, not direct mental health therapy
CMT IT	Social Media Integration	How organizations integrate social media-related topics into their curriculum or support services.	Session formats; Direct instruction and modules; Session topics; One-off interventions or spontaneous discussions; digital literacy tools; reframing exercises	3-session structure (body image, cyberbullying, narrative)
CMT	Assessment Methods	Tools and strategies used to identify participant needs, including surveys and observations.	Surveys; informal check-ins; partner-supplied reports	Triannual surveys; mentor check-ins
RDT IT	Key Strategic Partnerships	Types of partnerships (institutional, community-based, professional) that support the mission.	Schools, CBOs, therapists, universities; collaboration levels	Teachers, counselors, school administrators

RDT	Stakeholder Influence and External Pressures	Influence from donors, government policies, schools, and community partners. Degree and nature of influence exerted by funders or grant conditions on programming.	Government/policy priorities; Board and partner norms; Alignment to grant deliverables; content restrictions or freedom; Funder requirements;	Funders impact delivery scale but not content
IT	Institutional Pressures	Policies, regulations, or norms from relevant institutions (e.g., state, IRS, schools, boards, etc.) shaping nonprofit decisions or practices.	Adherence to or resistance from institutional norms/boards;	Board input on programs; alignment with SEL norms
CMT	Change Implementation Strategies	Internal strategies for adapting or evolving programming in response to contextual shifts. Methods used to adapt, revise, or improve programs based on internal evaluation and stakeholder feedback.	Surveys, observations, partnerships; one-on-one mentorship; Continuous evaluation; Iterative design; Relationship-building strategies	Responsive design; trauma-informed SEL
CMT	Challenges in adapting the curriculum	Obstacles or resistance encountered in program design or implementation, in general. Organizational barriers or tensions when addressing social media and identity.	Staffing and infrastructure constraints; Program scalability	Student pushback on identity guidance
CMT RDT	Best Practices & Future Opportunities	Promising practices, lessons learned, or areas for future investment and development. Suggestions or insights for enhancing the curriculum and programming in the future.	Curriculum shareability; participatory research; tech access; Classroom management; Facilitator training; Community and trust building	Trust-building; mentor-based implementation

Note: *RDT refers to Resource Dependency Theory; **IT refers to Institutional Theory; ***CMT refers to Change Management Theory